

CONSENT WAS NEVER THE POINT

*Alberta's Eugenic Inheritance and Canada's Institutional
Rebuild
1928 — Present*

The Alberta Disability System Breakdown

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About this document

Consent Was Never the Point is a long-form documentation piece produced by The Alberta Disability System Breakdown, an independent public advocacy campaign documenting the mandatory transition of approximately 79,290 Albertans from the Assured Income for the Severely Handicapped (AISH) program to the Alberta Disability Assistance Program (ADAP), effective July 1, 2026.

The campaign operates as a documentation-first public record. It is not a support group, a service provider, or a political advocacy organization in the conventional sense. Its work is to assemble, source, and make publicly available the legislative, fiscal, and structural record of the AISH-to-ADAP transition and the policy framework within which it has been undertaken.

This piece situates the contemporary record within a longer history. It is not a piece *about* Indigenous or disabled communities. It is a piece that draws from and amplifies the voices that have articulated this analysis from within those communities. Citations are provided throughout. The principal academic and primary-source references are listed in the bibliography.

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It is the same principle as being physically sterilized without consent simply because those who made the decision feel they have the right to decide what's best for those not consulted first.

A Note on This Work

This piece traces a single operating principle across six distinct historical and contemporary expressions: the explicit eugenic sterilization program established by *The Sexual Sterilization Act* (1928–1972); the structural-eugenic framework that persisted in policy after the Act’s repeal; the mandatory transition of disabled Albertans from AISH to ADAP under *Bill 12* (2025); the information asymmetry condition that prevents meaningful consent across all of the above; the contemporary rebuild of institutional infrastructure under *the Compassionate Intervention Act*; and the documented cross-Canada expansion of that institutional rebuild across Alberta, Saskatchewan, British Columbia, Manitoba, Ontario, and New Brunswick. The argument that follows is that these six expressions are not separate phenomena. They are six expressions of the same operating premise.

The argument is that the surgical mechanism of the historical eugenic period was repealed in 1972 and the institutional architecture was partially dismantled in the decades that followed. The operating premise that informed that period — that disabled people’s intimate, relational, and material lives are an appropriate site for government regulation in ways non-disabled people’s lives are not — was not specifically repudiated. It continued to operate through quieter means. It is now resurfacing in legislative form across multiple Canadian jurisdictions and is the subject of formal political demand in others. The pattern is not Alberta-isolated. The pattern is continental.

This edition of the piece is being prepared for submission to the United Nations Special Rapporteur on the Rights of Persons with Disabilities, Ms. Heba Hagrass (Egypt), appointed by the Human Rights Council on 1 November 2023. The reporting framework applied throughout is the Convention on the Rights of Persons with Disabilities, opened for signature December 13, 2006; signed by Canada March 30, 2007; ratified by Canada March 11, 2010. The piece engages, in particular, Articles 4(3), 12, 14, 19, 21, and 28, and CRPD General Comment No. 1 (2014) on Article 12 and General Comment No. 7 (2018) on Article 4(3) and Article 33(3).

What this piece is not. It is not a polemic; the historical material is more powerful than any rhetorical framing the writer could add. It is not a comparison that diminishes the historical victims of explicit eugenic violence. The argument is that the framework continues to operate, not that contemporary disabled Albertans are experiencing what Leilani Muir experienced. The continuity is structural, not equivalent in degree of harm. It is not, finally, an argument that any individual policy designer or government employee is a eugenicist. The argument is structural and operates through inheritance of operating premises rather than personal intent.

Coalition review. This piece has been read in draft by reviewers whose names and qualifications are not disclosed at their request, including at least one disability scholar familiar with the relevant academic literature, one reviewer with substantive harm-reduction policy expertise, and one Indigenous reader for the free, prior, and informed consent material in the chapters that follow. Final responsibility for the analysis rests with the campaign.

Audience. The intended audience is institutional — oversight bodies, academic disability studies, journalists, elected representatives willing to engage — together with disability community members who have felt the dynamic this piece names and the secondary audience of allied advocates in Indigenous rights, harm reduction, and consultation-failure contexts. A plain-language companion edition follows separately.

The piece is long. It is built that way because the argument cannot be made briefly without diminishing the historical material it draws on. The reader who knows the names of Leilani Muir, Catherine Frazee, and Heidi Janz will recognize the lineage. The reader who does not know those names will encounter them here. The piece closes where the title begins.

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Chapter I

The Sterilization Table

Alberta's Sexual Sterilization Act, 1928–1999

The 1928 Passage

On March 21, 1928, the Legislative Assembly of Alberta passed *The Sexual Sterilization Act*. The bill was introduced by George Hoadley, Minister of Health, in the Sixth Legislature of Alberta. It received Royal Assent the same day.¹

The Act created a four-member Alberta Eugenics Board. Two members were medical practitioners, nominated by the Senate of the University of Alberta and the Council of the College of Physicians and Surgeons. Two were non-medical members appointed by the Lieutenant Governor in Council. The Board's authority was to examine persons recommended for discharge from designated provincial institutions and to authorize sterilization, in the language of the Act's first section, "if the danger of procreation with its attendant risk of multiplication of the evil by transmission of the disability to progeny were eliminated."²

The original Act required either the consent of the patient or, where the patient was deemed incapable, the consent of a spouse, parent, or legal guardian. Board rulings had to be unanimous. Physicians and surgeons performing sterilizations under board order were granted immunity from civil and criminal liability.³

The Act met immediate opposition. In March 1928, a group calling itself the People's League to Act formed, retaining counsel and arguing publicly that the legislation was unconstitutional. Several hundred members joined; their petition did not delay Royal Assent.⁴

Alberta was the first jurisdiction in Canada — and the only jurisdiction in the British Empire — to enact eugenic sterilization legislation that was vigorously implemented and sustained. British Columbia would follow in 1933 with a parallel act of substantially smaller scale, performing fewer than two hundred documented sterilizations. No other province passed comparable legislation.⁵

The Operating Mechanism

The Act functioned through an institutional pipeline. The Provincial Mental Hospital at Ponoka, the Mental Hospital at Oliver near Edmonton, and — increasingly through the 1940s — the Provincial Training School for Mental Defectives at Red Deer provided the population from which the Eugenics Board drew its candidates.

The Provincial Training School was the institution that came to define the program's operation. Opened in October 1923 in a converted residential college building on the Michener hill, it housed children and adults designated "mental defectives." At its peak it held two hundred residents — approximately one fifth of the population of the city of Red Deer at the time. The institution accounted for roughly one quarter of all cases brought before the Eugenics Board over the Act's forty-four years of operation.⁶

A network of provincial guidance clinics, established from the 1930s onward, screened children for referral to the institution. The clinics served the eugenics program in two practical ways: they identified candidates before institutionalization, reducing costs; and they generated case files that documented the social and family circumstances later used as evidence before the board.⁷

In 1937, the Legislative Assembly amended the Act. The amendment removed the requirement of consent for persons classified as "mentally defective." Before 1937, the consent requirement had functioned as a brake on the program's pace. The board had approved 1,283 candidates for sterilization between 1928 and 1938, of whom 644 had been sterilized. After 1937, the rate of completed sterilizations rose sharply.⁸

A second amendment followed in 1942, expanding the diagnostic categories under which sterilization could be authorized.⁹

Over the Act's full operating period from 1928 to 1972, the Alberta Eugenics Board considered 4,725 cases. Of those, 2,834 sterilizations were performed.¹⁰

The population on whom these operations were performed was not demographically representative of Alberta. According to the most detailed academic survey of the records — Jana Grekul, Harvey Krahn, and Dave Odynak's 2004 analysis in the *Journal of Historical Sociology* — 64.7 percent of those sterilized were women. 70.6 percent were under twenty years of age at the time of sterilization. 49.8 percent lived in rural communities. 40.6 percent were unemployed at the time of their referral. 25.7 percent were identified in the records as Métis or Indian, against a Métis and Indigenous share of the provincial population at the time of approximately 3.4 percent.¹¹

The Indigenous overrepresentation intensified in the program's final years. Researchers examining records from 1969 to 1972 — the four years immediately preceding repeal — have documented Indigenous representation averaging approximately twenty-five percent of all sterilizations performed in those years.¹²

The Provincial Training School in Red Deer was supervised for the last twenty-five years of the Act's operation by Leonard Jan Le Vann, superintendent from 1949 to 1974. Le Vann's role combined the duties of institutional administration with the assessment of individual residents for referral to the Eugenics Board. The board's chair throughout most of the program's operation was John M. MacEachran, founder of the Department of Philosophy and Psychology at the University of Alberta, who served as chairman from 1929 to 1965.¹³

The shift away from these institutions had already begun. The 1969 Alberta Mental Health Study, the Blair Report, urged an end to large-scale institutional care; Alberta then recorded the steepest psychiatric-bed reduction in Canada, from 4.1 beds per thousand in 1965 to 0.7 by 1980.

The 1972 Repeal

In 1972, the newly elected government of Premier Peter Lougheed repealed *The Sexual Sterilization Act*. The repeal followed forty-four years of operation and brought formal eugenic sterilization legislation in Canada to an end.¹⁴

What the repeal accomplished is precisely defined. The legal framework authorizing the Alberta Eugenics Board, and the immunity granted to performing physicians, was removed. The institutional pipeline — the Provincial Training School at Red Deer, the guidance clinics, the network of feeder facilities — remained.

The Provincial Training School was renamed the Michener Centre in 1977, in honour of the recently retired Governor General Roland Michener. By 1983 the renamed institution had expanded to sixty-six buildings on a 360-acre site. Its resident population was reduced over the subsequent decades through deinstitutionalization, group-home alternatives, and policy reform; the Alberta government announced its closure in 2013.¹⁵

The repeal of the Act did not by itself include any public accounting of who had been sterilized, on what records, or with what continuing harm. No survivor was contacted. No records were released. The operating premise that the Eugenics Board had been empowered to act on — that the reproductive lives of disabled adults were an appropriate site for institutional decision-making in ways non-disabled adults' lives were not — was not addressed in the repealing legislation or in any government statement at the time.

For the next twenty-three years, that was the status of the matter.

Muir v. Alberta (1996)

Leilani Marietta Muir was born in 1944. Her childhood was characterized in the later trial record by neglect and physical abuse. In 1955, at the age of ten, she was admitted by her mother to the Provincial Training School at Red Deer.¹⁶

The trial judgment of Madam Justice Joanne B. Veit found that the admission process did not establish that Muir met the legal definition of “mentally defective” that the institution’s enabling legislation required. The court found that she had been wrongly labelled a “moron” — a clinical category in use at the time — and that her ten-year confinement, from age ten to age twenty-one, was wrongful in law.¹⁷

In 1959, at the age of fourteen, Muir was brought before the Eugenics Board. The board’s order authorizing her sterilization was signed by John MacEachran, then chairman. The surgical sterilization was performed shortly thereafter. The procedure was irreversible.¹⁸

Muir left the institution in 1965, at the age of twenty-one. She later married, attempted to start a family, and learned through medical investigation that she had been sterilized. Subsequent attempts to adopt were also unsuccessful.

In 1993, she filed suit against the Province of Alberta. The case went to trial in 1995. The Province acknowledged its obligation to pay damages and left to the court the determination of how much.¹⁹

Justice Veit's judgment, dated January 25, 1996, awarded \$250,280 in general damages for pain and suffering from the sterilization — the statutory maximum for such damages at the time — and an additional \$125,000 in aggravated damages for the wrongful stigmatization of Muir as a "moron, a high grade mental defective." With prejudgment interest, the total monetary award was \$740,780. The Province was further ordered to pay \$230,000 in legal costs.²⁰

The judgment's central finding was articulated in language that has been quoted in nearly every academic and journalistic account since. Justice Veit wrote:

"The circumstances of Ms. Muir's sterilization were so high-handed and so contemptuous of the statutory authority to effect sterilization, and were undertaken in an atmosphere that so little respected Ms. Muir's human dignity that the community's, and the court's, sense of decency is offended."²¹

The decision did not stand only on Muir's individual case. The court reviewed the historical record of the Eugenics Board's operations, the legislative framework, and the institutional culture at the Provincial Training School. The judgment functioned, in effect, as the public record's first formal judicial recognition that the program had operated outside the limits of its own enabling statute — that "many things done in the name of eugenics were clearly illegal," in the formulation of one academic commentator on the trial.²²

Muir's victory opened the door to additional litigation. By 1998, more than seven hundred surviving claimants had filed suit against the Province.²³

Bill 26 and the Notwithstanding Clause (1998)

The provincial response to those filings was Bill 26, the *Institutional Confinement and Sexual Sterilization Compensation Act*, introduced in the second session of the twenty-fourth Legislature in March 1998 by the government of Premier Ralph Klein.

Bill 26 proposed two operative measures. First, it capped damages payable to victims of wrongful institutional confinement and sterilization at \$150,000 per person. Second, it imposed a 180-day limitation period after which no further actions could be commenced.²⁴

Section 3 of the bill was the unprecedented element. It read:

"This Act operates notwithstanding (a) the provisions of sections 2 and 7 to 15 of the Canadian Charter of Rights and Freedoms, Part 1, Constitution Act, 1982, Schedule B to the Canada Act, 1982, (U.K.) 1982 c11, and (b) the Alberta Bill of Rights."²⁵

The sections of the Charter referenced — sections 2 and 7 through 15 — encompass fundamental freedoms, the right to life, liberty, and security of the person, the right against unreasonable search and seizure, the right not to be arbitrarily detained, and the equality rights guaranteed under section 15. Section 33 of the Charter, the "notwithstanding clause," permits a provincial legislature to declare that an Act shall operate notwithstanding those guarantees for a renewable period of five years.

Bill 26 represented the first attempt by an Alberta government to invoke section 33.²⁶

The bill was introduced on the afternoon of one legislative sitting day. The reaction was immediate and widely covered. Within twenty-four hours, the Klein government announced its withdrawal. Justice Minister Jon Havelock told reporters that the surviving victims would retain the right to sue the Province for damages, and that out-of-court settlements would be pursued. Premier Klein, defending the bill before its withdrawal, told reporters that it had been intended to protect taxpayers from lawsuits potentially totalling \$760 million.²⁷

In the period following the withdrawal, the Province negotiated settlements with the great majority of surviving claimants. Approximately five hundred individuals settled in 1998 for a combined total of \$48 million. An additional 247 individuals reached settlement in 1999 for a combined total of \$82 million. At the 1999 announcement, Justice Minister David Hancock — then in the portfolio — apologized only after a reporter’s prompting. Premier Klein did not attend, and offered when asked: “We extend regrets for the actions of another government in another period of time.”²⁸

What This Chapter Establishes

The legal framework of explicit eugenic sterilization in Alberta is documented and acknowledged. *The Sexual Sterilization Act* was passed in 1928, amended in 1937 and 1942 to expand its reach and remove consent, operated through a centralized board for forty-four years, sterilized 2,834 people — disproportionately women, young people, the poor, and the Indigenous population — and was repealed in 1972. The Province has paid approximately \$130 million in settlements to surviving victims and one judicially decided plaintiff.

That is the record. The chapters that follow build on it.

But three observations about the record are necessary before this chapter closes.

The first concerns the structure of the response. When the *Muir* judgment opened the possibility of broader liability, the first instinct of the Alberta government was to invoke section 33 of the Charter to suspend the very rights — to equality, to security of the person, to access to justice — that the original Act had violated. The bill was withdrawn within twenty-four hours, not because the government recognized the contradiction, but because of public reaction. The recognition of the contradiction has, on the public record, never been made.

The second concerns the framing. Premier Klein’s “another government in another period of time” formulation places the wrongdoing safely in a prior administration. The government in 1998 was not the government in 1928; the government in 1972 — which repealed the Act — was a Progressive Conservative administration with substantial continuity into the Klein government. The chapters that follow will examine, in concrete contemporary instances, the question of whether a government can be said to have repudiated a framework whose institutional and policy inheritance it never inventoried.

The third concerns the operating premise. The legal authority to sterilize disabled people without their consent was removed in 1972. The legal authority to make decisions about disabled people’s lives —

about their housing, their income, their relationships, their access to medical care, and their freedom from involuntary institutionalization — on the basis that the deciding authority knows best, was not removed. It was not addressed. The question of whether that premise had any place in a constitutional democracy was, on the public record, never asked.

Subsequent chapters address that question.

Notes — Chapter I

1. *The Sexual Sterilization Act*, SA 1928, c 37; Legislative Assembly of Alberta, Sixth Legislature, Second Session; Royal Assent March 21, 1928. See also “Alberta passes Sexual Sterilization Act,” Living Archives on Eugenics in Western Canada (Eugenics Archive Canada), timeline entry, accessed May 22, 2026.
2. *The Sexual Sterilization Act*, SA 1928, c 37, s 5(1). The Eugenics Board composition is set out in sections 2 and 3 of the Act. The text quoted appears in section 5 governing the Board’s authority over discharge candidates.
3. *The Sexual Sterilization Act*, SA 1928, c 37, ss 6, 7. Section 6 governed consent; section 7 granted immunity from civil and criminal liability.
4. Erika Dyck, *Facing Eugenics: Reproduction, Sterilization, and the Politics of Choice* (Toronto: University of Toronto Press, 2013), ch. 2; Angus McLaren, *Our Own Master Race: Eugenics in Canada, 1885–1945* (Toronto: McClelland & Stewart, 1990).
5. Douglas Wahlsten, “Leilani Muir versus the Philosopher King: Eugenics on Trial in Alberta,” *Genetica* 99 (1997); Dyck, *Facing Eugenics*. British Columbia enacted its *Sexual Sterilization Act* in 1933; estimates of sterilizations performed under it range below 200.
6. “Provincial Training School opens in Red Deer, Alberta,” Eugenics Archive Canada, timeline entry, accessed May 22, 2026; City of Red Deer Archives, Michener Centre collection. The Provincial Training School building was originally the Alberta Ladies’ College of Red Deer (1913), converted in 1916 to a sanatorium for shell-shocked soldiers and reorganized in 1923 to house disabled children and adults under provincial supervision.
7. “Guidance Clinics,” Eugenics Archive Canada, encyclopedia entry, accessed May 22, 2026.
8. Statutes of Alberta, 1937 (amending *The Sexual Sterilization Act*). Numerical figures are drawn from Jana Grekul, Harvey Krahn, and Dave Odynak, “Sterilizing the ‘Feeble-Minded’: Eugenics in Alberta, 1929–1972,” *Journal of Historical Sociology* 17, no. 4 (2004): 358–384.
9. Statutes of Alberta, 1942 (second amendment to *The Sexual Sterilization Act*); consolidated as *The Sexual Sterilization Act*, RSA 1942, c 194. The 1942 amendment expanded the diagnostic categories under which sterilization could be authorized.
10. The Canadian Encyclopedia, “Red Deer,” accessed May 22, 2026; Eugenics Archive Canada, Alberta Eugenics Board records. Numerical sources vary slightly: 4,725 cases considered (Canadian Encyclopedia, “Red Deer”); 4,739 cases presented (Karen Stote, “Sterilization of Indigenous Women in Canada,” *Canadian Encyclopedia*). The figure of 2,834 sterilizations performed is consistent across sources.
11. Grekul, Krahn, and Odynak, “Sterilizing the ‘Feeble-Minded’.” The 25.7 percent figure for Métis and Indigenous representation is drawn from the case-file analysis of all available Eugenics Board records undertaken by Grekul and colleagues at the University of Alberta.
12. Karen Stote, “Sterilization of Indigenous Women in Canada,” *The Canadian Encyclopedia*, accessed May 22, 2026. The 1969–1972 figure derives from analysis of the case records preserved by the Provincial Archives of Alberta.
13. Wahlsten, “Leilani Muir versus the Philosopher King”; “Provincial Training School,” Eugenics Archive Canada. MacEachran was a founding member of the University of Alberta faculty and served on the Eugenics Board from its inception until his retirement in 1965. Le Vann resigned as PTS Superintendent in 1974, two years after the Act’s repeal.
14. Statutes of Alberta, 1972 (repealing *The Sexual Sterilization Act*). The Lougheed Progressive Conservatives had been elected to government in August 1971, displacing the Social Credit administration that had held power in Alberta since 1935.
15. City of Red Deer Archives, Michener Centre collection; Government of Alberta, “Government announces closure of Michener Centre,” news release, March 11, 2013. The closure was partially reversed in subsequent years; the

institution's remaining residents remained on the site under reduced operations.

16. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. (background of plaintiff and circumstances of admission to the Provincial Training School).
17. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (findings of wrongful confinement and wrongful labelling).
18. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (sterilization order, irreversibility, and consent analysis); Wahlsten, "Leilani Muir versus the Philosopher King" (MacEachran's signature on the sterilization order).
19. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J.: "In 1959, the Province wrongfully surgically sterilized Ms. Muir and now acknowledges its obligation to pay damages to her. However, the Province leaves to the court the determination of how much the Province should pay."
20. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (general damages, aggravated damages, prejudgment interest, and costs).
21. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. The quoted passage is the most frequently cited line in the judgment and appears in the early section of Justice Veit's reasons.
22. Wahlsten, "Leilani Muir versus the Philosopher King."
23. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The CBC report identifies "700 surviving victims" with active claims at the time of the Bill 26 introduction.
24. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, 2nd Session, 24th Legislature, Alberta, 1998 (introduced and withdrawn), ss 1, 4.
25. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, s 3 ("Override of Charter").
26. "Alberta and the Notwithstanding Clause," *LawNow* (Centre for Public Legal Education Alberta), 1998; Senator Paula Simons, Intervention 655944-74, Senate of Canada, accessed May 22, 2026. Both confirm Bill 26 as the first attempt by an Alberta government to invoke section 33 of the *Charter*.
27. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The \$760 million estimate attributed to Klein was reported in this CBC story and in contemporaneous coverage by other outlets.
28. The Globe and Mail, "Leilani Muir made history suing Alberta over forced sterilization," obituary, March 16, 2016 (settlement totals and Klein quotation); CBC News, "Alberta compensates sterilization victims," November 14, 1998. The 1998 settlement covered approximately 500 claimants; the 1999 supplemental settlement covered an additional 247.

Chapter II

The Definitional Framework

*Eugenics, Structural Eugenics, and the Consent Standard***Eugenics: The Scholarly Definition**

Eugenics is not a term that has been retired from contemporary scholarly use. The U.S. National Human Genome Research Institute (NHGRI), the federal research institution that led the public-sector contribution to the Human Genome Project and that continues to anchor American genomics policy, maintains an active definition in its public-facing Talking Glossary of Genomic and Genetic Terms: “Eugenics is a discredited belief that selective breeding for certain inherited human traits can improve the ‘fitness’ of future generations.”¹ In its parallel Fact Sheets series, NHGRI describes eugenics as “the scientifically erroneous and immoral theory of ‘racial improvement’ and ‘planned breeding’.”²

The institutional position is consistent. Eugenics is identified by the NHGRI both as a coherent historical theory and as a persistent contemporary phenomenon. The agency’s 2021 symposium *The Meaning of Eugenics: Historical and Present-Day Discussions of Eugenics and Scientific Racism*, co-hosted with the U.S. National Library of Medicine, framed the contemporary relevance directly: addressing the history of eugenics, NHGRI wrote, “is important because it has persisted in policies and beliefs around the world, including the United States.”³

The definitional point that matters most to the argument of this piece is the scope of the mechanisms eugenics has historically employed. The Alberta record in Chapter I documented six. They operated together: surgical sterilization under *The Sexual Sterilization Act*; institutional segregation through facilities such as the Provincial Training School at Red Deer; the marriage prohibitions and immigration exclusions of the same period; the cohabitation restrictions enforced within residential institutions; and the active provincial discouragement, through institutional policy and through guidance-clinic referrals, of disabled people forming households and families.

The 1972 repeal of *The Sexual Sterilization Act* addressed the first of these. The legal authority to surgically sterilize disabled people without their consent was removed. The other mechanisms — institutional segregation, restrictions on partnership and family formation, the structural exclusion of disabled people from full community participation — were not removed by the repealing legislation. They were addressed, where addressed at all, through subsequent and unrelated policy reform (deinstitutionalization, group-home alternatives, human-rights legislation against discrimination on the basis of disability), each of which proceeded on its own timeline and with its own incompleteness.

The structure of contemporary disability policy in Alberta, treated in detail in Chapter III, retains substantial portions of the second through sixth mechanisms — not in their historical form, and not under

their historical names, but in their structural effect. The chapter that follows establishes the vocabulary by which that retention is recognized in the disability studies literature.

Structural Eugenics

The term “structural eugenics” — sometimes “newgenics” in the formulation of philosopher Robert A. Wilson — describes a documented phenomenon in disability studies scholarship: policies that are designed for stated non-eugenic reasons (cost containment, bureaucratic uniformity, fiscal restraint, employment promotion) and that nevertheless produce effects historically associated with explicit eugenic programs (prevention of disabled reproduction, institutional or quasi-institutional segregation, exclusion from full social and economic participation), and that are sustained politically because they produce those effects without anyone in the policy-design or political-implementation processes being required to defend them as such.⁴

Robert Wilson, who directed the SSHRC-funded Living Archives on Eugenics in Western Canada project at the University of Alberta and built the EugenicsArchive.ca documentation system referenced throughout Chapter I, develops this analysis at length in *The Eugenic Mind Project* (MIT Press, 2018). Wilson’s central proposition is that “eugenic thinking” — what he calls the “eugenic mind” — outlasted the legal frameworks of the explicit eugenic period and continues to operate in contemporary practice. The mechanisms he identifies include the conceptualization of certain human traits as eugenic concerns, the systematic construction of categories of normality against which non-normality is judged, and what he names the “sub-humanizing” of populations who fall outside those categories.⁵

The disability studies literature on contemporary policy follows this analytical thread. Nancy E. Hansen, Heidi L. Janz, and Dick J. Sobsey, in a 2008 article in *The Lancet*, wrote: “There are disturbing similarities between Nazi arguments concerning ‘quality of life’, ‘useless eaters’, or ‘lives less worthy’ and discussions of disability currently taking place among ‘mainstream’ geneticists and bioethicists advocating a value scale of humanness.”⁶ The point Hansen, Janz, and Sobsey make is not that contemporary bioethicists are Nazis. It is that the logical structure of a “value scale of humanness” — the proposition that some lives are less worth living than others, and that this assessment can be made by external authority — is the same structure historical eugenics built itself on, and that this structure has not been retired.

Catherine Frazee, professor emerita in the School of Disability Studies at Toronto Metropolitan University and former Chief Commissioner of the Ontario Human Rights Commission, has articulated the same analysis in the context of Canada’s Medical Assistance in Dying (MAID) framework. In her work with Heidi Janz on the GRIM (Grievous and Irremediable) Project — a documentation initiative gathering stories of disabled Canadians who have pursued MAID under the post-Bill-C-7 expansion — Frazee and Janz argue that the structural availability of state-administered death for disabled people who are not dying, combined with the structural unavailability of adequate income, housing, attendant care, and disability support, constitutes a modern form of eugenics.⁷ As Frazee has articulated the argument: the suffering driving MAID requests by disabled applicants is, in many of the cases the GRIM Project documents, suffering produced by social and material conditions the state has the capacity to alter and has

chosen not to. The logical structure mirrors the historical eugenic argument that institutionalization was protective rather than coercive: the modern claim that MAID expansion is compassionate is, in the GRIM analysis, indistinguishable in structure from the historical claim that sterilization was therapeutic.

The intent claim is not the structural-eugenics claim. The piece this chapter is part of is not arguing that contemporary policy designers or government employees are eugenicists. The argument is that some policies produce eugenic effects — measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to access the material conditions of a livable life — and that those effects are sustained in policy because the political infrastructure for producing and defending them does not require anyone to name them.

The intent claim is the harder claim. The effect claim is what the literature supports. The piece works from the effect claim.

The Consent Standard — CRPD Article 4(3)

A binding international standard governs the development of disability policy in Canada. The *Convention on the Rights of Persons with Disabilities* (CRPD) was adopted by the United Nations General Assembly on December 13, 2006, opened for signature on March 30, 2007, and entered into force on May 3, 2008. Canada signed the Convention on the day it opened for signature and ratified it on March 11, 2010. Canada acceded to the Convention’s Optional Protocol on December 3, 2018.⁸

Article 4(3) of the Convention, located in the general obligations provision and therefore applicable transversally across the treaty, reads in full:

In the development and implementation of legislation and policies to implement the present Convention, and in other decision-making processes concerning issues relating to persons with disabilities, States Parties shall closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations.⁹

The text contains four operative components. First, the obligation attaches to “the development and implementation” of legislation and policies — that is, to the upstream design phase and the downstream operational phase, not only to the moment of legislative consideration. Second, it extends to “other decision-making processes concerning issues relating to persons with disabilities” — that is, beyond the legislative branch to administrative and executive decision-making. Third, the obligation is to “closely consult with and actively involve” — language requiring substantive, ongoing engagement rather than notification or post-hoc comment. Fourth, the consultation must be carried out “through their representative organizations” — that is, with the organizations that the disability community itself recognizes as representative, not with individual stakeholders selected by the consulting authority.

In November 2018, the UN Committee on the Rights of Persons with Disabilities — the treaty body established under the Convention to monitor State Party implementation — issued General Comment No. 7, providing authoritative guidance on the application of Article 4(3). The General Comment characterizes the participatory requirement of the Convention as a signal obligation that stands out among core human

rights conventions, reflecting the “Nothing About Us Without Us” principle that the international disability community has carried into the treaty’s framework.¹⁰

Article 4(3) is not a domestic political norm. It is a treaty obligation that Canada has voluntarily assumed and that binds the Government of Canada and, through the constitutional division of powers, the implementation of laws and policies within provincial jurisdiction that engage the rights protected under the Convention.

The piece returns to the application of this standard to *Bill 12* in Chapter IV.

The Parallel Standard — FPIC under UNDRIP

The structural problem CRPD Article 4(3) addresses — the imposition of legislation on a defined population without that population’s meaningful participation in advance of the imposition — is not unique to disability policy. The same problem governs the relationship between settler states and Indigenous Peoples whose lands and rights are affected by state action. The international framework developed in response is the doctrine of free, prior, and informed consent (FPIC), articulated most directly in the *United Nations Declaration on the Rights of Indigenous Peoples* (UNDRIP), adopted by the UN General Assembly on September 13, 2007.¹¹

FPIC appears in six articles of UNDRIP, governing six distinct categories of state action: Article 10 (the relocation of Indigenous Peoples from their lands or territories); Article 11 (cultural, intellectual, religious, and spiritual property); Article 19 (the adoption and implementation of legislative or administrative measures); Article 28 (restitution for lands taken without consent); Article 29(2) (the storage or disposal of hazardous materials on Indigenous lands); and Article 32(2) (project approvals affecting territory and resources).¹²

Article 19, the article most directly parallel to CRPD Article 4(3) in its scope and operation, reads:

States shall consult and cooperate in good faith with the Indigenous Peoples concerned through their own representative institutions in order to obtain their free, prior and informed consent before adopting and implementing legislative or administrative measures that may affect them.¹³

The structural parallel between Article 19 of UNDRIP and Article 4(3) of the CRPD is exact. Both attach to the development and implementation of legislative and administrative measures that affect the relevant population. Both require consultation with representative institutions or representative organizations of that population. Both situate the consultation obligation in the upstream design phase rather than the downstream comment phase. Both have been understood by their respective treaty bodies as substantive engagement requirements rather than notification requirements.

Canada has incorporated UNDRIP into federal domestic law through the *United Nations Declaration on the Rights of Indigenous Peoples Act*, SC 2021, c 14. The federal framework legislation requires that federal laws be brought into conformity with the Declaration and that an action plan be developed to implement its principles. Provincial governments are not directly bound by the federal Act, but the

principle that legislative and administrative measures affecting Indigenous Peoples require their free, prior, and informed consent operates as a recognized international norm regardless of domestic implementing legislation.¹⁴

In July 2025, the Government of Alberta provided a concrete illustration of how this norm is being treated at the provincial level. On July 3, 2025, Alberta’s Minister of Environment and Protected Areas, Rebecca Schulz, joined with Ontario’s Minister of the Environment, Todd McCarthy, in a public letter to the federal Minister of Indigenous Services. The letter requested that the federal government refrain from reintroducing Bill C-61, the *First Nations Clean Water Act*, originally tabled in December 2023 and died on the Order Paper at the March 2025 prorogation of Parliament. Bill C-61 would have, among other things, affirmed First Nations’ inherent right to self-government and jurisdiction over water resources on First Nation lands.¹⁵

The Chiefs of the Blackfoot Confederacy — the Kainai, Siksika, and Piikani Nations — responded on July 4, 2025 with a statement characterizing Alberta’s opposition as “morally bankrupt” and stating that “Premier Smith’s government opposes First Nations’ rights to safe and sufficient water supplies, which is utterly reprehensible.”¹⁶

The structural point recurring in this chapter is that the Alberta government was, during the same period in 2025 in which it was opposing Bill C-61 at the federal level, advancing Bill 12 at the provincial level. The same provincial administration was, in successive months of 2025, declining to support legislation that would have implemented the FPIC standard for First Nations and advancing legislation that did not meet the CRPD Article 4(3) standard for disabled Albertans. Neither population had been consulted, in advance of the action being taken, through the representative institutions or organizations that international treaty standards require.

What This Chapter Establishes

Three propositions follow from this chapter, and the chapters that follow draw on them without restating.

First, eugenics as a phenomenon is broader than surgical sterilization. The institutional position of the National Human Genome Research Institute, the standard scholarly position in the historical literature, and the operating definition of the disability studies field all encompass institutional segregation, marriage and immigration restrictions, prevention of family formation, and what Robert Wilson names the “sub-humanizing” of disabled populations. The 1972 repeal of *The Sexual Sterilization Act* addressed one of these mechanisms. The others were not repudiated by the repealing legislation and have not been repudiated since.

Second, “structural eugenics” is a documented term in disability studies for the contemporary form taken by mechanisms that historically operated under explicit eugenic justification. The intent claim is the harder claim and is not the claim this piece makes. The effect claim — that contemporary disability policy produces measurable reductions in disabled people’s capacity to form households, to reproduce, to maintain community, and to access the material conditions of a livable life — is what the literature supports, and what the chapters that follow document at the level of specific Alberta policy.

Third, two binding international standards require consultation with affected populations through their representative organizations before legislative and administrative measures are adopted. CRPD Article 4(3) governs the development and implementation of disability policy. FPIC under UNDRIP Articles 10, 11, 19, 28, 29, and 32 governs the development and implementation of measures affecting Indigenous Peoples. The Government of Alberta has, within a six-month window in 2025, failed both standards: by opposing federal legislation (Bill C-61) that would have implemented the FPIC standard for First Nations, and by advancing provincial legislation (Bill 12) that did not meet the CRPD Article 4(3) standard for disabled Albertans.

The chapters that follow apply this framework to the inventory of Alberta disability policy: cohabitation rules, guardianship structures, institutional care practices, sub-survival income, the FSCD three-year wait, the Canada Disability Benefit clawback, and the Compassionate Intervention Centres now under construction.

The mechanism has changed. The principle has not.

Notes — Chapter II

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Chapter III

Structural Eugenics in Contemporary Alberta

Six Mechanisms in the Contemporary Architecture

Chapter II established the analytic framework: eugenics is broader than surgical sterilization; “structural eugenics” is the documented disability-studies term for policies producing eugenic effects without requiring eugenic intent; and consultation under CRPD Article 4(3) is a binding international standard that Alberta has not met in the development of contemporary disability policy. This chapter applies that framework to six specific Alberta policies currently in force or scheduled to come into force in 2026. The inventory is not exhaustive. The six mechanisms documented here are those for which the primary-source record is the most complete and the structural-eugenics mapping is the most direct.

Each section identifies the policy, the documented effect, and the historical eugenic mechanism it parallels. The analytic claim is the effect claim defined in Chapter II: that these policies produce measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to maintain long-term partnerships, and to access the material conditions of a livable life. The intent claim is not the claim made here. The effect claim is what the documented record supports.

The Cohabitation Rule

Under current AISH policy, the income of any adult sharing a household with an AISH recipient may be assessed as part of the household income calculation and used to reduce the recipient’s benefit. The rule applies regardless of the nature of the relationship: a married spouse, a common-law partner, a roommate sharing rent, an adult child temporarily living at home, an elderly parent in a granny suite, and a co-parenting partner who is not a romantic partner are all treated under the same household-income provision. The rule contains narrow exemptions, notably for paid live-in caregivers under documented arrangements; the exemptions do not cover practical roommate, co-parenting, or multi-generational household arrangements common in low-income, Indigenous, and immigrant communities.¹

On May 12, 2026, the Government of Alberta filed AR 96/2026, a new General Regulation governing both AISH and ADAP, in force on July 2, 2026. Section 8(4) of the regulation introduces what the government’s fact sheet describes as an “allowance for two-client couples living together”: where two adults in the same household each receive disability income assistance, each partner receives 88 percent of the maximum individual benefit. The reduction applies regardless of which program each partner is on.²

The mathematics, drawn from the government's own base rates, are as follows. Two AISH recipients sharing a household lose \$5,587.20 per year against the baseline of two unreduced AISH benefits. A mixed AISH-and-ADAP household loses \$7,699.20 per year. Two ADAP recipients lose \$9,811.20 per year. These figures are calculated before any employment income is considered and before the federal Canada Disability Benefit clawback discussed in section 3 of this chapter, which compounds against the same household.³

The structural effect is documented and direct. Partnership between disabled adults receiving AISH or ADAP is, after July 2, 2026, a \$5,587 to \$9,811 annual penalty against the household. Practical household-sharing arrangements between an AISH recipient and any other adult — romantic, familial, platonic — place the recipient's benefit at risk of reduction through the broader cohabitation rule. The recipient is, in the language of Inclusion Alberta's ADAP fact sheet, "structurally incentivized to remain alone."⁴

The historical eugenic mechanism the rule parallels is direct. The marriage prohibitions, immigration exclusions, cohabitation restrictions in residential institutions, and active provincial discouragement of family formation by disabled people documented in Chapter I operated through legal prohibition. The contemporary mechanism operates through financial penalty. The legal prohibition has been repealed; the financial mechanism that produces equivalent isolation has not been.

Sub-Survival Income: AISH, ADAP, and the Housing Crisis

AISH provides a maximum living allowance of \$1,940 per month in 2026. ADAP, the program to which all AISH recipients without severe and profound developmental disabilities, PDD eligibility, or palliative/terminal conditions will be transitioned on July 1, 2026, provides a maximum of \$1,740 per month. Recipients who are transitioned receive a transition payment maintaining the AISH rate until December 31, 2027; on January 1, 2028, the payment drops to the ADAP rate.⁵

Average one-bedroom rent in Edmonton and Calgary ranged from \$1,500 to \$1,900 in 2024, with documented year-over-year increases of 17.5 percent province-wide and up to 21.3 percent in three-bedroom markets, against a provincial average of \$2,138 per month for three-bedroom units. Alberta does not impose a residential rent-increase cap.⁶

The arithmetic produces the documented condition. A single AISH recipient on the maximum living allowance at the lower end of one-bedroom rent has, after rent, between forty and four hundred forty dollars per month for all other costs — food, utilities, transportation, medical out-of-pocket expenses, clothing, household supplies. Statistics Canada and Canada Food Price Report estimates for single-adult grocery costs in 2026 are eight hundred to one thousand dollars per month. Alberta utility estimates are three hundred eighty-five to five hundred ten dollars per month. The single recipient on private-market rent is operating, by these calculations, at a structural deficit of seven hundred fifty dollars or more per month.⁷

In Alberta's 2023–2024 fiscal year, 83.6 percent of AISH recipients had no employment income. The Government of Alberta's framing of ADAP as an empowerment program enabling Albertans with

disabilities to pursue employment operates against a documented baseline in which the great majority of recipients are not, on the available evidence, in a position to do so.⁸

The historical eugenic mechanism the income structure parallels is institutional segregation. The historical residential institutions documented in Chapter I — the Provincial Training School at Red Deer and the network of mental hospitals and guidance clinics — excluded disabled people from full community participation by removing them from the community. The contemporary income structure excludes disabled people from full community participation by setting income at a level that does not permit the housing, food, transportation, or social engagement that participation requires. The mechanism has changed from institutional confinement to economic confinement. The effect — exclusion from the conditions of community life — is the same.

The Canada Disability Benefit Clawback

The federal *Canada Disability Benefit Act* was passed in 2023 as a poverty-reduction measure for working-age Canadians with disabilities. The benefit pays up to \$200 per month, subject to a Disability Tax Credit prerequisite and an income-tested eligibility framework. The federal government, in introducing the benefit, explicitly requested that provinces and territories not reduce their own disability income payments when recipients also receive the federal benefit.⁹

In a letter to AISH recipients dated July 2, 2025, the Government of Alberta notified recipients that the Canada Disability Benefit would be treated as non-exempt income under AISH and ADAP. Recipients who did not demonstrate that they had applied for the CDB would see an equivalent \$200 deduction from their provincial benefits. Alberta is the only province or territory in Canada to claw back the Canada Disability Benefit dollar for dollar. Every other province and territory fully exempts the federal payment.¹⁰

The fiscal effect is direct. Applied to approximately 79,290 AISH recipients at \$200 per month, the clawback captures approximately \$190 million per year in federal transfer payments that would, in any other Canadian province, be received by disabled recipients. In Alberta, the federal investment is redirected to the provincial budget; the net benefit to the disabled recipient is zero.¹¹

The structural effect is to nullify federal poverty-reduction policy at the provincial level for the recipient population the federal policy was designed to serve. The historical eugenic mechanism the clawback parallels is the active provincial discouragement of disabled people from accessing resources available to non-disabled people. The historical version operated through institutional segregation. The contemporary version operates through the structural capture of federal transfer payments designed to lift disabled Albertans above the poverty line.

The FSCD Three-Year Wait

The Family Support for Children with Disabilities program funds respite, in-home support, therapy services, and equipment for Alberta children under eighteen with severe disabilities. It is the primary provincial mechanism through which families of disabled children access intervention services during the developmental years.¹²

In October and November 2024, Inclusion Alberta surveyed 540 families from 79 of Alberta's 87 electoral districts about their FSCD experience. The findings, published in January 2025, document an average wait of three years from application to full range of FSCD services. Eighty-five percent of families who had received the partial Family Support Services agreement that follows initial eligibility were still without a caseworker for the full Child Focused Services component. Fifty-eight percent of families reported negative effects from going without supports, including job loss, missed developmental windows, and measurable physical and mental health deterioration.¹³

The Auditor General of Alberta examined FSCD in 2022 and reported that caseworker training and oversight processes were inadequate and that families “may not receive the supports and services they qualify for.” The Government of Alberta received those findings in 2022. Caseworker caseloads have since been documented at four hundred families per worker against a standard reference of one hundred. Seven temporary caseworker contracts were not renewed in September 2024. The FSCD budget increase in Budget 2025 was 3.61 percent against a documented population-plus-inflation requirement of 7.3 percent — a real-terms reduction of approximately 3.69 percentage points.¹⁴

The structural effect is documented in the developmental literature on early intervention. The years between roughly two and five years of age constitute the period in which intervention services for developmental disability produce the largest measurable gains in functional outcomes. A three-year wait from application moves children past this window before services begin. A child diagnosed at four whose family applies the same day will not, on the documented average, receive full FSCD services until age seven — outside the window the program's own evidence base identifies as most consequential.¹⁵

The historical eugenic mechanism the FSCD wait parallels is the institutional intervention in disabled children's developmental trajectory documented in Chapter I. The historical version operated through institutionalization at the Provincial Training School and similar facilities, where children were separated from family and community on the asserted ground that institutional care was protective. The contemporary version operates through administrative wait time. The mechanism has changed from confinement to delay. The effect — the developmental trajectory the disabled child would otherwise have had is not the trajectory they receive — is the same.

PDD: Guardianship, Trusteeship, and the Unreported Waitlist

The Persons with Developmental Disabilities program supports adults eighteen and over with developmental disabilities of onset before age eighteen. It funds residential support, employment support, community access, and individualized care planning. PDD currently supports more than thirteen thousand adults in Alberta. The waitlist — the number of adults who are eligible for PDD but not receiving services — has not been publicly reported by the Government of Alberta in three years. The last published waitlist data are from 2021.¹⁶

Inclusion Alberta surveyed 540 PDD-connected individuals in late 2024 specifically because the Government would not release waitlist data. The findings, published in January 2025, document that seventy percent of respondents still need to meet with a caseworker to develop the outcome plan that determines their services. Forty-one percent of those who do receive supports report that the supports are

not helping. The PDD budget increase in Budget 2025 was 5.94 percent against the 7.3 percent population-plus-inflation requirement — a real-terms reduction of approximately 1.36 percentage points.¹⁷

The structural framework PDD operates within is the *Alberta Adult Guardianship and Trusteeship Act*. The Act establishes a substitute-decision-making framework for adults whose capacity to make personal and financial decisions is operationally impaired. The Act's framework conflicts with Article 12 (equal recognition before the law) of the CRPD, which the UN Committee on the Rights of Persons with Disabilities has identified, in General Comment No. 1, as requiring transition from substitute decision-making to supported decision-making across signatory jurisdictions.¹⁸

The historical eugenic mechanism the PDD framework parallels is the assertion of institutional authority over the lives of disabled adults. The Alberta Eugenics Board documented in Chapter I asserted authority to determine that disabled adults' reproductive lives were an appropriate site for provincial decision-making. The contemporary AGTA framework asserts authority over disabled adults' personal-and-financial decisions through a substitute-decision-making model that international human rights law has identified as requiring reform. The mechanism has changed from sterilization authority to substitute-decision-making authority. The operating premise — that disabled adults are appropriate subjects of external decision-making in ways non-disabled adults are not — is the operating premise of both periods.

Institutional Care: The Separation of Couples

The Alberta continuing care system, encompassing long-term care facilities (Continuing Care Home Type A) and designated supportive living facilities (Continuing Care Home Type B), operates with documented bed-availability constraints, facility-specialization patterns that distribute different levels of care across distinct facilities, and a fee-determination architecture based on couple's combined-income calculations. The combination produces a documented pattern in which spouses are forced to live apart when one requires institutional care that the other does not.¹⁹

In June 2019, CBC News reported the case of Martha and Willard Farnell of Calgary, Alberta. The Farnells had been married for sixty-six years. Following Willard's dementia diagnosis and admission to a long-term care facility approximately twenty minutes' drive from Martha's residence, the Alberta continuing care system did not provide a mechanism through which the couple could continue to share accommodation. The CBC report documented Sweden's policy position treating forced spousal separation in long-term care as inhumane, and the Netherlands "Living Apart Together" suites model that permits couples to live in separate but connected rooms or apartments within the same care facility.²⁰

The peer-reviewed medical literature documents that severe loneliness affects up to forty-two percent of long-term-care residents, against ten percent prevalence in older adults living in the community. The mortality and morbidity consequences of social isolation in this population are documented at scale.²¹

Ontario's long-term care regulatory framework, since January 1, 2018, has required every long-term care home in the province to have at least two beds available for spouses. Alberta has no equivalent requirement. The provincial Government's own acknowledged capacity need is ten thousand new continuing-care spaces over three years; documented commitments are at a small fraction of that figure.²²

The historical eugenic mechanism the institutional-care separation pattern parallels is direct. The cohabitation restrictions enforced in the historical residential institutions documented in Chapter I — the Provincial Training School, the network of mental hospitals — separated disabled adults from spouses, family, and community as a matter of institutional policy. The contemporary version separates senior spouses, one or both of whom is disabled, through bed-allocation and facility-specialization mechanisms that the policy architecture characterizes as administrative necessity. The mechanism has changed from policy prohibition to administrative allocation. The effect — partnered disabled adults are separated from their partners by the institutional system at the moment they most need continued partnership — is the same.

The Missing Middle: A Bimodal Support System

The six mechanisms inventoried above describe specific structural features of Alberta’s disability framework. A seventh structural feature requires its own treatment because it organizes how the others are distributed: the province’s disability supports are bimodally structured. There are intensive supports available to persons at the most severely disabled end of the spectrum — group homes, residential PDD placements, institutional care — and there are minimal accommodations available at the most functional end — workplace duty-to-accommodate, post-secondary disability services, employment-readiness programs. The substantial cohort between those two ends — persons with moderate intellectual disability, persons with fluctuating mental illness, persons with learning disabilities, persons with attention and executive-function disabilities, persons with cognitive impairments that do not require twenty-four hour care but that do require case-by-case adjustment to access employment, education, housing, and services — is the missing middle. The supports available to this cohort are systematically inadequate to their case-by-case needs.²³

The empirical proportion. The most recent Statistics Canada Canadian Survey on Disability, conducted in 2022 and released in December 2023, finds that fifty-nine percent of Canadians with disabilities have what Statistics Canada classifies as “milder” disabilities — mild or moderate — and forty-one percent have “more severe” disabilities — severe or very severe. The moderate cohort, in the Statistics Canada data structure, is part of the milder grouping. The missing middle is not a marginal sub-population. It is the majority of the disabled population.²⁴

The employment record. The same Survey’s 2017 data, broken down by severity, shows employment rates of 76 percent for persons with mild disabilities, 67 percent for moderate, 49 percent for severe, and 31 percent for very severe. The 76 percent rate at the mild end is close to the rate for the general population (80 percent in the comparable period). The 31 percent rate at the very severe end is the population for which institutional or high-intensity supports exist. The 67 percent rate for the moderate cohort — the missing middle — is the population that is most often expected to participate in mainstream employment on the basis of minimal accommodation, but that does not consistently achieve parity with non-disabled employment outcomes. The gap between expected participation and actual participation is the missing middle’s structural location.²⁵

The case-by-case nature of the need. What distinguishes the missing middle as a structural matter is not the average level of impairment but the case-by-case variability of the supports required. A person with severe intellectual disability whose support needs are continuous and predictable can be served by a group-home placement and a permanent funding stream. A person with mild dyslexia whose support needs are at the level of access to text-to-speech and additional exam time can be served by a duty-to-accommodate process that the employer or institution administers. The person whose needs change month by month, whose functional capacity depends on a fluctuating mental-health presentation, whose access to an employment opportunity depends on a particular modification that is unlike the modification needed for the next opportunity — that person requires case-by-case adjudication by a person who knows their situation. The Alberta system has historically supplied that case-by-case adjudication, where it has been supplied at all, through individual caseworkers.²⁶

The FSCD caseworker reduction. The Family Support for Children with Disabilities program has been the principal childhood-disability case-management mechanism in Alberta. Section 4 of this chapter documents that the wait time from FSCD application to full service has reached an average of three years; that caseloads at four hundred families per worker substantially exceed the standard reference of one hundred; that seven temporary caseworker contracts were not renewed in September 2024; and that the FSCD budget increase in Budget 2025 was a real-terms reduction. The cumulative direction of the program is away from individual caseworker relationships and toward online intake, partial Family Support Services agreements that do not include a caseworker, and longer waits for the case-by-case adjudication the missing middle's children require. Hold My Hand Alberta, the parent advocacy initiative co-founded by Keltie Marshall, has been on the public record across 2024 and 2025 documenting these effects in family-by-family detail.²⁷

AISH and ADAP as the adult-side analogue. The AISH-to-ADAP transition under Bill 12, examined at length in Chapter IV, is the adult-side analogue of the FSCD caseworker reduction. Eighty thousand AISH recipients, the substantial majority of whom have cognitive, mental, or combined cognitive-and-mental presentations, are being moved into a program (ADAP) that is structured around the expectation of employment readiness with assistance, rather than around the recognition that case-by-case adjudication of moderate disability requires sustained professional case management. The transition removes the right of appeal that previously permitted a recipient to contest a denial of supports. The structural location is the same as the FSCD location: the missing middle is being moved out of the case-by-case adjudicative framework and into a more automated, less individualized, less appealable framework.²⁸

The two-tier outcome. The bimodal structure produces a documented two-tier outcome. Persons at the high-severity end retain access to structured supports (though not at adequate levels). Persons at the low-severity end retain access to accommodation processes that the duty to accommodate ensures cannot be entirely withdrawn without legal exposure. The missing middle does not have either. It does not qualify for the structured supports because it is not severe enough; the structured supports are not designed for moderate need. It does not achieve effective access to the accommodation processes because case-by-case adjudication has been progressively withdrawn; the processes assume a self-advocating recipient who can navigate complex bureaucratic systems, which is the precise capacity moderate cognitive or mental-health disability often impairs. The missing middle falls between.²⁹

The connection to the institutional rebuild. The population that the contemporary institutional-rebuild framework (Chapter VI) most often names as its target — persons with combined mental-health, substance-use, and cognitive-impairment presentations — is the same population as the missing middle. It is not the severely disabled cohort with twenty-four hour care needs. It is not the mildly disabled cohort whose accommodation needs are routine. It is the moderate-disability cohort whose case-by-case needs have not been adequately served through case-by-case adjudication and who now appear in the framework’s named target as persons whose situation has deteriorated to the point of requiring involuntary intervention. The structural failure to serve the missing middle on a case-by-case basis is, in this analysis, an upstream condition of the institutional rebuild’s target population. The framework captures the population the structural framework has failed.³⁰

The historical parallel. The 1928–1972 Sexual Sterilization Act period applied disproportionately to persons classified as “mentally defective” — a category that, in the Eugenics Board’s operational application, captured a substantial cohort of persons whose presentations would today be described as mild to moderate intellectual disability, learning disability, or borderline cognitive functioning combined with social marginalization. The category was not, in the Board’s caseload, dominated by persons with severe twenty-four hour care needs. It was dominated by persons whom the institutional framework of the period could not be made to serve adequately on a case-by-case basis. The documented record of the missing middle in 2026 is a description of the same structural position, in a different vocabulary, in a different framework, at a different historical moment.³¹

What This Chapter Establishes

The six mechanisms inventoried in this chapter are not the complete inventory of structural eugenics in contemporary Alberta. They are the inventory for which the primary-source record is the most complete. Each is documented in current Alberta legislation, regulation, or program policy. Each is verified against current government data or against survey work conducted by Inclusion Alberta, the Auditor General of Alberta, or the peer-reviewed medical literature. None of the six requires speculative inference; each operates as documented.

Three observations follow.

First, the six mechanisms operate together. The cohabitation rule restricts partnership. Sub-survival income restricts community participation. The CDB clawback nullifies federal poverty-reduction intervention. The FSCD wait restricts developmental trajectory. PDD guardianship restricts adult autonomy. Institutional care separation restricts continued partnership in aging. Taken together, they constitute an architecture in which disabled people are structurally separated from the relational, developmental, financial, and social conditions of full life across the lifecycle. The architecture is not the product of a single decision. It is the cumulative product of decisions taken over decades, each defensible in isolation, none requiring eugenic intent.

Second, the historical mechanisms documented in Chapter I have direct parallels in this inventory. The marriage prohibitions of the 1928–1972 period are paralleled by the cohabitation rule. The institutional segregation of the Provincial Training School period is paralleled by the income and housing structure that

prevents community-based independent living. The institutional cohabitation restrictions are paralleled by the long-term-care separation pattern. The institutional authority over disabled adults' lives asserted by the Eugenics Board is paralleled by the Adult Guardianship and Trusteeship Act substitute-decision-making framework. The developmental intervention of the historical institutionalization period is paralleled by the FSCD wait. The active discouragement of disabled family formation documented in the 1928–1972 record is paralleled by the integrated effect of all six contemporary mechanisms.

Third, none of the six mechanisms documented here was developed through a consultation process meeting the standard CRPD Article 4(3) requires. The cohabitation rule was filed by regulation on May 12, 2026, ten days after the Inclusion Alberta media release identifying Bill 12 as eroding disability rights, and was not preceded by consultation with disability community representative organizations on its specific 88-percent mechanism. The CDB clawback was communicated to recipients by letter on July 2, 2025, without consultation. The Bill 12 framework producing the AISH-to-ADAP transition was passed on December 11, 2025 over the documented opposition of every major Alberta disability advocacy organization. The FSCD, PDD, and institutional-care frameworks operate within budget and policy cycles that do not, on the available record, include the consultation through representative organizations the Convention requires.

The mechanism has changed. The architecture has not.

Notes — Chapter III

1. Government of Alberta, Assured Income for the Severely Handicapped Regulation, prior framework at Alta Reg 91/2007 (repealed and replaced May 2026); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, sections on the cohabitation rule and the isolation trap. The narrow exemption for paid live-in caregivers does not extend to practical roommate, co-parenting, or multi-generational household arrangements.
2. Assured Income for the Severely Handicapped General Regulation, AR 96/2026, filed May 12, 2026, in force July 2, 2026, section 8(4); Government of Alberta, Assisted Living and Social Services, “Alberta Disability Assistance Program Fact Sheet,” May 12, 2026.
3. Calculations based on the Government of Alberta’s published AISH base rate (\$1,940/month) and ADAP base rate (\$1,740/month) for 2026; see The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, for full worked calculations of dual-recipient household income losses under AR 96/2026 section 8(4).
4. Inclusion Alberta, *ADAP Fact Sheet*, December 2025; see also The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section on the isolation trap.
5. Government of Alberta, AISH and ADAP program documentation, 2025–2026; The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, on the transition payment expiring December 31, 2027.
6. Canada Mortgage and Housing Corporation, Rental Market Report 2024; Urbanation/Rentals.ca, National Rent Report, June 2024 (17.5 percent Alberta year-over-year increase); GIL Property Management, “Alberta Rent Prices in 2024,” November 2024 (three-bedroom average \$2,138/month, 21.3 percent year-over-year increase). Alberta does not impose a residential rent-increase cap; British Columbia, by comparison, set its 2026 limit at 2.3 percent.
7. Dalhousie University, *Canada’s Food Price Report 2026*, December 2025; Alberta utility cost estimates compiled in The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, sections on the math of housing affordability.
8. Government of Alberta, AISH program data, 2023–2024 fiscal year; Inclusion Alberta, *ADAP Fact Sheet*, December 2025.
9. *Canada Disability Benefit Act*, SC 2023, c 9; Employment and Social Development Canada, Canada Disability Benefit program documentation, 2025.
10. Government of Alberta letter to AISH recipients, July 2, 2025 (notification of CDB treatment as non-exempt income); CBC News, “Federal disability benefit to be deducted from AISH payments,” Bellefontaine, M., February 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Gillian Petit, “The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis,” *Canadian Tax Observatory*, April 22, 2026.
11. Calculations based on Government of Alberta AISH Caseload Open Data (approximately 79,290 recipients as of September 2025) and the \$200/month maximum CDB payment.
12. Government of Alberta, Family Support for Children with Disabilities program documentation, alberta.ca.
13. Inclusion Alberta, *Experiences with Family Support for Children with Disabilities* (FSCD Survey Report), January 2025. 540 respondents from 79 of 87 Alberta electoral districts.
14. Auditor General of Alberta, *Report on FSCD*, 2022 (caseworker training and oversight findings); Hold My Hand Alberta / The Albertan News, “Some Alberta caseworkers that help disabled children have 100s of clients,” September 16, 2024; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
15. The developmental early-intervention window for children between approximately ages two and five is documented across the developmental disability literature. See, e.g., discussions in the FSCD Survey Report (Inclusion Alberta, January 2025) and the broader developmental pediatrics literature on intervention timing.

16. Inclusion Alberta / ALIGN, *PDD Current State: Critical and Urgent: Experiences with Persons with Developmental Disabilities*, January 2025; CBC News, “Provincial support delays are hurting disabled Albertans and families, reports find,” Bellefontaine, M., January 23, 2025.
17. Inclusion Alberta / ALIGN, *PDD Current State*; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
18. *Adult Guardianship and Trusteeship Act*, Statutes of Alberta, 2008 (provincial framework for substitute decision-making and trusteeship arrangements); Committee on the Rights of Persons with Disabilities, General Comment No. 1 (2014) on Article 12 of the CRPD (equal recognition before the law), document CRPD/C/GC/1.
19. Government of Alberta, continuing-care framework, including *Continuing Care Act* and related regulations; The Alberta Disability System Breakdown, *Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act*, Part One, Section 1.10.
20. Melissa Mancini, “Separating Couples in Long-Term Care: Some Nations Call It Inhumane, in Most of Canada It’s Routine,” CBC News, June 2, 2019, <https://www.cbc.ca/news/health/national-long-term-care-couples-housing-1.5159102>. The CBC report documents the Farnell case and comparator-jurisdiction approaches.
21. Joanna Briggs Institute, documentation of severe loneliness prevalence in long-term-care residents (up to 42 percent) compared to community-dwelling older adults (10 percent); see *Building Up*, Part One, Section 1.10, for the documented health-and-stability cascade.
22. Ontario long-term care spousal-beds regulation, in force January 1, 2018; Government of Alberta acknowledged capacity need of 10,000 new continuing-care spaces over three years, per the March 2024 funding announcement (Big West Country, “Alberta’s Government Announces Funding for Improved Access to Healthcare,” March 15, 2024).
23. The bimodal-structure framing draws on the disability policy literature distinguishing intensive institutional supports, case-by-case adjudicative supports, and accommodation-based supports. The framing is developed in the Cardus policy paper “Breaking Down Work Barriers for People with Disabilities” (May 2023), cardus.ca, which documents the bimodal allocation of federal and provincial disability spending between income assistance and employment supports.
24. Statistics Canada, Canadian Survey on Disability 2022, released through The Daily on December 1, 2023, www150.statcan.gc.ca/n1/daily-quotidien/231201/dq231201b-eng.htm. Employment severity breakdown reported from Statistics Canada Canadian Survey on Disability 2017, as analyzed by Cardus (op. cit.) and by the Labour Market Information Council (lmic-cimt.ca, “Where’s the data about disability and the labour market in Canada?”).
25. See note 24 above. The case-by-case-adjudication distinction is the analytic contribution of this section; it builds on the disability policy literature on supported decision-making (CRPD General Comment No. 1 (2014) on Article 12, UN Doc CRPD/C/GC/1, paragraphs 16–19) and on the social-model literature on accommodation-by-accommodation versus case-by-case adjudication.
26. See note 13 above (Inclusion Alberta FSCD survey). Coverage of caseworker reductions: St. Albert Gazette (Brett McKay, Local Journalism Initiative Reporter), “Staff cuts will affect hundreds of families of kids with disabilities, parents say,” September 16, 2024; stalbertgazette.com. Statements of Keltie Marshall, co-founder of Hold My Hand Alberta (holdmyhandalberta).
27. Cross-reference to Chapter IV, sections addressing Bill 12 and the removal of appeal rights under the AISH-to-ADAP transition. AISH caseload composition data (cognitive, mental, combined): Government of Alberta AISH program statistics, September 2025 caseload of 79,290; 22,419 cognitive (28.3 percent); 23,940 mental illness (30.2 percent); 46,359 combined (58.5 percent).
28. The two-tier outcome is documented in the employment-quality literature for persons with disabilities. See Lero, Pletsch, and Hilbrecht, “The Employment Quality of Persons with Disabilities: Findings from a National Survey,” *Journal of Occupational Rehabilitation* (2023), [pmc.ncbi.nlm.nih.gov/articles/PMC10090748/](https://pubmed.ncbi.nlm.nih.gov/articles/PMC10090748/), which documents

that persons with combined physical and mental/cognitive conditions are at the highest risk of either “instrumental” (secure but trapped) or “precarious” (insecure and unrewarding) employment outcomes.

29. The capture-by-failed-supports analysis connecting the missing middle to the institutional rebuild’s target population is developed in Chapter VI (Modern Eugenics Momentum) and Chapter VIII (The Cross-Canada Record). The target-population descriptions in the BC, Alberta, and Saskatchewan frameworks all identify combined mental-health, substance-use, and cognitive-impairment presentations as the named target.
30. Cross-reference to Chapter I, sections on the 1937 amendment to the Sexual Sterilization Act (SA 1937, c 47) and on Eugenics Board caseload composition. The historical-categorical parallel is developed at greater length in Chapter VII.
31. Footnote reserved for cross-reference to the closing section of this chapter.

Chapter IV

The AISH-to-ADAP Transition and the Consultation Standard

Bill 12, the Premier's Council, and the UN Committee Warning

The Transition

On December 9, 2025, the Alberta Legislative Assembly passed Bill 12, the *Financial Statutes Amendment Act, 2025 (No. 2)*. The bill received Royal Assent on December 11, 2025 and was enacted as SA 2025 c20.¹

Bill 12 effects what Inclusion Alberta has described as the most consequential restructuring of disability income in Alberta in a generation. Effective July 1, 2026, all current AISH recipients will be transitioned to the Alberta Disability Assistance Program (ADAP). The transition affects approximately 79,290 Albertans — the population the Government of Alberta has formally assessed as having severe and permanent disabilities preventing employment. Section 12.8 of Bill 12 removes the right of appeal for the transition decision. Recipients who wish to remain on AISH must reapply through a Medical Review Panel whose decisions are, on the legislation's own terms, final and not subject to appeal to an independent body.²

This chapter does not catalogue the substantive content of Bill 12 again; the structural mechanisms it activates — sub-survival income, cohabitation rules, employment-exemption reductions, federal-transfer capture — are documented in Chapter III. This chapter is concerned with a different question: whether the procedural framework under which Bill 12 was developed and passed met the consultation standard Canada has formally accepted under the United Nations Convention on the Rights of Persons with Disabilities. The conclusion the documented record supports is that it did not.

The Government's Consultation Claim

The Government of Alberta has, throughout the period of Bill 12's development and passage, advanced a single foundational rhetorical claim about ADAP. The claim appears on the program's public information page, in the May 12, 2026 Fact Sheet, and in ministerial correspondence to AISH recipients and to municipal councils. It reads:

ADAP is being thoughtfully designed, based on input from Albertans with disabilities...
ADAP public engagement concluded September 12, 2025. Thousands of Albertans participated in engagement via roundtables, telephone town halls, written submissions and

the online survey. Public engagement was undertaken in addition to engagement with disability advocates and service providers, including through roundtables and meetings.³

The structure of the rhetorical claim has three components. First, the program reflects what Albertans with disabilities asked for. Second, the engagement was numerically substantial — “thousands of Albertans.” Third, the engagement was augmented by direct meetings with advocates and service providers. Every government defence of Bill 12 against criticism — including responses to municipal motions, to media inquiry, and to written correspondence from constituents — rests on this foundation.⁴

What the Consultation Standard Requires

Chapter II of this piece establishes the binding international framework. CRPD Article 4(3), to which Canada has been bound since ratification on March 11, 2010, requires State Parties to “closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations” in the development and implementation of legislation and policies concerning persons with disabilities. The CRPD Committee’s General Comment No. 7 (2018) elaborates the standard. Consultation must be substantive and ongoing rather than notification-based. It must occur in the design phase, not only the comment phase. It must be conducted through representative organizations the disability community itself recognizes — not through individuals selected by the consulting authority. And it must, as a matter of substantive engagement, produce outcomes that the affected population can reasonably recognize as reflecting their input.⁵

Alberta’s domestic framework for meeting this standard has a specific institutional embodiment. The *Premier’s Council on the Status of Persons with Disabilities Act*, RSA 2000, c P-21, establishes the Premier’s Council as the standing statutory advisory body for disability policy. The Council is required by statute to advise on the development of disability legislation and programs. Its structure includes specific subcommittees for particular populations, including Indigenous people with disabilities. Its consultation processes, even when conducted by online survey, are framed as advisory input feeding into a multi-stakeholder analytic process led by disability community members themselves.⁶

The question this chapter examines is whether the consultation process undertaken by the Government of Alberta in connection with Bill 12 met the standard the Convention requires and the standard the province’s own statutory framework contemplates. The documented record establishes that it did not.

The Documented Gaps Between the Claim and the Standard

The Premier’s Council bypass. The Government of Alberta did not, in its public-facing consultation materials or in its responses to municipal advocacy, route the ADAP consultation through the Premier’s Council. The Council’s 2023–2026 Strategic Plan survey, conducted through its standing statutory framework, received 381 responses. The ADAP consultation, conducted through a parallel bespoke process the government directly controlled, is reported to have drawn “thousands.” The disparity reflects where the government directed participation effort: away from the standing mechanism, toward the process the government itself designed. The Premier’s Council exists. Its statute requires it to advise on the development of disability policy. The government chose to route ADAP consultation outside the Council. The choice itself is a documentation gap; a government that relied on its standing statutory body for this consultation would have a defensible structural answer to many of the further concerns documented below.⁷

The undisaggregated “thousands.” The government’s claim that “thousands of Albertans” participated in the consultation is not, on the public record, broken down by recipient status. The campaign has formally requested the participant breakdown: current AISH recipients, current recipients of other disability benefits, family members and caregivers of recipients, service providers and advocates, non-recipient Albertans with disabilities, and other. The government has not provided this breakdown. The Convention’s standard requires consultation with persons with disabilities through their representative organizations; the consultation cannot be verified as meeting that standard without data on who, specifically, participated.⁸

Provisions introduced after consultation closed. The consultation window closed on September 12, 2025 (a date the engagement page subsequently and quietly changed to September 14, 2025). Bill 12 was introduced on November 25, 2025, more than two months after the consultation closed. Key operative provisions of Bill 12 — including the removal of independent appeal rights through the abolition of the Citizens Appeal Panel, and the substantial revision of employment income exemption figures from the levels indicated in the August 2025 Discussion Guide — were not in the consultation materials on which participants based their input. The CRPD Article 4(3) standard requires consultation on the substantive content of legislation. Consultation on a discussion guide whose provisions are subsequently revised does not, on its face, meet that standard.⁹

The petition tabled at the Legislature. In October 2025, Calgary-based advocate Darryl Learie initiated a petition against the ADAP framework. The petition was circulated through disability community networks across Alberta and was tabled at the Alberta Legislature with documented signatures. The petition constitutes a documented empirical statement, by named Albertans with disabilities and their families, against the program the government’s consultation claim says reflects their input. A consultation process producing a program against which the population it consulted petitions does not, on its face, reflect what that population asked for.¹⁰

The opposition record across the disability sector. Inclusion Alberta’s November 26, 2025 media release stated that “Bill 12 erodes rights and deepens poverty for Albertans with disabilities.” The Alberta Medical Association, the Alberta Union of Provincial Employees, Disability Action Hall, the People’s

Alliance for Disabled Albertans, MS Canada, and Friends of Medicare Lethbridge have publicly opposed Bill 12. The Chair of Friends of Medicare Lethbridge, Bev Muendel-Atherstone, has characterized the transition as “the equivalent of being found guilty of fraud or malingering without having a trial or sentencing.” No major Alberta disability advocacy organization has publicly endorsed the Bill 12 framework. A municipal coalition including Edmonton, Calgary, Lethbridge, Camrose, Claresholm, Red Deer, St. Albert, and Medicine Hat has called for the AISH-to-ADAP transition to be paused pending meaningful consultation.¹¹

The engagement page record. The Alberta government’s ADAP engagement page — the public-facing documentation of the consultation the government’s rhetorical claim rests on — was rewritten from an active consultation portal to a look-back archive within seven days of the consultation closing. The page’s stated survey end date was quietly changed from September 12 (the date in the Discussion Guide and on the page during the consultation) to September 14, 2025. The page sat unchanged for 206 days through the introduction of Bill 12, the passage of Bill 12, and the March 2026 budget announcement — none of which were added to the page that documents the consultation feeding into those milestones. Since April 12, 2026, the page has been blocked from public archiving via the Wayback Machine. The pattern is documented in the campaign’s *Engagement Page Record: A Forensic Timeline*.¹²

The May 21, 2026 cabinet shuffle. On May 21, 2026 — nine days after the ADAP regulations were filed without the eligibility criteria, and during the week the individualized eligibility-determination letters began arriving with the 80,000-recipient AISH caseload — Premier Danielle Smith shuffled the provincial cabinet following the resignations of Finance Minister Nate Horner and Hospitals Minister Matt Jones. Jason Nixon, the Minister responsible for the AISH-to-ADAP transition through its design, the December 2025 passage of Bill 12, the engagement-page record, and the May 12, 2026 regulation filing, moved to Finance and President of Treasury Board. Lethbridge-East MLA Nathan Neudorf took over the Assisted Living and Social Services portfolio. Minister Neudorf inherits operational responsibility for the AISH-to-ADAP transition five weeks before the July 1, 2026 launch and one week into the individualized-eligibility-letter wave. The consultation record this section documents was generated under one minister; the implementation will be administered, beginning five weeks before launch, under a different minister. The continuity of the consultation record is, consequently, also a question.^{12a}

The UN Committee Warning, March 18, 2025

On March 18, 2025, nearly nine months before Bill 12’s passage in the Alberta Legislative Assembly, the United Nations Committee on the Rights of Persons with Disabilities issued its Concluding Observations on the combined second and third periodic reports of Canada. The Committee, in its capacity as the treaty body monitoring CRPD implementation, examined the operation of Canadian disability law and policy across federal, provincial, and territorial jurisdictions.¹³

Among the Committee’s specific recommendations, one is directly relevant to the structural circumstance under which Bill 12 was developed and passed. The Committee recommended that Canada “establish legally defined mechanisms ensuring compliance with the CRPD by all provinces and territories, and condition fiscal transfers from the Federal level to provinces and territories on their adherence to the

CRPD.”¹⁴

The Committee’s recommendation addresses, in operational terms, the constitutional reality that human rights treaties ratified by the federal government are not automatically binding on provincial governments in their areas of constitutional jurisdiction. The federal government ratified the Convention; provincial governments operate the substantial portion of disability policy directly affecting recipients. Where a province develops legislation in an area engaging the rights protected under the Convention without meeting the Convention’s consultation standard, federal mechanisms to ensure compliance become the mechanism by which Canada’s treaty obligations are operationalized at the provincial level.

Canada has not, on the documented public record, acted on the Committee’s recommendation. No federal mechanism exists to condition fiscal transfers to provinces on CRPD compliance. The province that bypassed its own standing statutory advisory body in the development of the most consequential disability legislation in a generation faced no federal-level operational consequence for doing so. The Committee’s March 18, 2025 warning of exactly this structural risk preceded Bill 12’s passage by nine months. Bill 12 proceeded.

The Removal of Appeal Rights

Bill 12’s most operationally consequential provision, after the transition itself, is its elimination of independent appeal rights for the affected population.

Under the AISH framework that existed before Bill 12, recipients who disagreed with an eligibility decision had recourse to the Citizens Appeal Panel — a quasi-judicial body composed of community members with relevant expertise, operating at arm’s length from the AISH program and from the Government of Alberta. The Panel’s decisions were binding on the program. The Alberta Ombudsman’s public-facing description confirmed the appeal route: “Most decisions about your benefits can be appealed through the Appeals Secretariat.”¹⁵

Bill 12 abolished the Citizens Appeal Panel as the appeal body for AISH eligibility decisions. In its place, the legislation establishes a Medical Review Panel composed of medical professionals appointed by the Government. The Medical Review Panel reviews disputed eligibility decisions. Its decisions are final. There is no independent appeal from a Medical Review Panel decision; judicial review remains available only on procedural grounds, not on the merits.¹⁶

Section 12.8 of Bill 12 specifically provides that the transition decision — the determination that an AISH recipient is reassigned to ADAP rather than retained on AISH — cannot be appealed. The CBC News reporting on August 21, 2025 quoted the ADAP Discussion Guide describing the Medical Review Panel as making decisions that “are final and not subject to appeal.”¹⁷

Bill 12 also gives the Government of Alberta authority to limit which other program decisions can be appealed under the post-transition framework. The appeal-rights structure that operated as the procedural protection for AISH recipients across decades no longer exists in the form it existed in as recently as 2024.¹⁸

The CRPD Article 13 protection of access to justice for persons with disabilities is implicated directly by this provision. So is the structural conflict of interest inherent in the new arrangement: the party with the financial interest in reducing the AISH caseload (the Government of Alberta) controls the panel that determines who stays on AISH (the Medical Review Panel), and the Medical Review Panel's decisions are final. The independent check that the Citizens Appeal Panel provided no longer exists.

The Parallel to 1998

Chapter I of this piece documented a previous legislative moment in Alberta in which the provincial government, faced with the fiscal implications of a court decision recognizing rights of persons with disabilities, responded by attempting to constrain those rights through legislation. In 1998, the Klein government's response to the wave of sterilization lawsuits opened by the *Muir* judgment was Bill 26 of that year, the *Institutional Confinement and Sexual Sterilization Compensation Act*, which attempted to invoke section 33 of the Charter to cap damages and impose a 180-day limitation period. The 1998 Bill 26 was withdrawn within twenty-four hours under public pressure. The underlying impulse — that the appropriate institutional response to a population's assertion of rights is the constraint of those rights — was, in that moment, defeated in public.¹⁹

Bill 12 in 2025 operates on the same impulse through different mechanisms. The 1998 bill would have invoked section 33 of the Charter explicitly. The 2025 framework operates without the visible constitutional override the 1998 bill required, but the structural effect is comparable: the rights protected under the CRPD — specifically Articles 4(3), 12, 13, 19, 21, and 28 — are not honoured in the operation of the legislation. The recipient population was not consulted through representative organizations in the development phase. The legal-capacity protections of Article 12 are not honoured by the reversal of 79,290 prior findings of permanent disability without individual reassessment. The access-to-justice protections of Article 13 are not honoured by the elimination of independent appeal. The community-living protections of Article 19 are not honoured by the income reduction to ADAP levels. The accessible-information protections of Article 21 are not honoured by the absence of plain-language documentation of program operational rules. The adequate-standard-of-living protections of Article 28 are not honoured by the transition to sub-survival benefit levels.²⁰

In 1998, the constraint of rights was attempted explicitly and failed. In 2025, the constraint of rights was effected administratively and succeeded. The 1998 effort had to be defended at the constitutional level. The 2025 effort proceeded without that requirement, because the operational mechanisms were located in the financial structure of the program rather than in an explicit Charter override. The structural outcome is the same. The procedural visibility is different.

What This Chapter Establishes

Four observations follow from this chapter.

First, the consultation undertaken by the Government of Alberta in connection with Bill 12 did not meet the standard CRPD Article 4(3) requires. The standing statutory advisory body was bypassed. The

participant breakdown supporting the “thousands of Albertans” claim has not been published. Key program provisions were introduced after consultation closed. A petition against the program was tabled at the Legislature. No major Alberta disability advocacy organization endorsed the framework. The engagement page itself has been altered in ways that are visible in the historical archive and that the government has not publicly explained.

Second, the international treaty body whose mandate is to monitor implementation of the Convention had specifically warned Canada about exactly this structural risk nine months before Bill 12 passed. The Committee’s recommendation of legally defined federal mechanisms to condition fiscal transfers on provincial CRPD compliance was issued on March 18, 2025. Bill 12 passed on December 9, 2025. The Committee’s warning was on the record. The provincial framework proceeded.

Third, the elimination of independent appeal under Bill 12 transfers final adjudicatory authority over AISH eligibility to a panel appointed by the same Government of Alberta that benefits financially from reducing the AISH caseload. The structural conflict of interest is built into the legislation. The procedural protection that the Citizens Appeal Panel provided for decades has been removed. The access-to-justice protections the Convention requires under Article 13 are not, on the documented record, honoured by this arrangement.

Fourth, the parallel to 1998 is not metaphorical. The Klein government’s 1998 Bill 26 attempted to constrain the rights of sterilization survivors through explicit Charter override; it was withdrawn under public pressure within twenty-four hours. The Smith government’s 2025 Bill 12 constrains the rights of contemporary disabled Albertans through administrative mechanisms that operate below the threshold of explicit constitutional confrontation. The structural impulse — that the appropriate institutional response to a population’s assertion of rights is the constraint of those rights — is the same. The visibility of the impulse is what has changed.

The chapter that follows examines the upstream condition without which the consultation standard cannot operate: the right of the affected population to know, in advance, what they are being asked to consent to.

Notes — Chapter IV

1. Legislative Assembly of Alberta, Bill 12, *Financial Statutes Amendment Act, 2025 (No. 2)*, 31st Legislature, Second Session; passed at Third Reading December 9, 2025; received Royal Assent December 11, 2025; enacted as SA 2025 c20. Bill text and legislative status available at the Legislative Assembly of Alberta document repository.
2. Bill 12, section 12.8 (removal of appeal of the AISH-to-ADAP transition); see also CBC News (Bellefontaine), August 21, 2025, reporting on the ADAP Discussion Guide; Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; Zachary Weeks, “Bill 12 and ADAP: What Alberta’s New Disability Law Means for AISH Recipients,” November 28, 2025. The 79,290 figure is from Government of Alberta, AISH Caseload Open Data, September 2025.
3. Government of Alberta, Alberta Disability Assistance Program public information page, alberta.ca/alberta-disability-assistance-program; Government of Alberta, Assisted Living and Social Services, ADAP Fact Sheet, May 12, 2026. The same foundational consultation claim appears across both sources.
4. See, for example, Minister Jason Nixon’s correspondence to AISH recipients (including the April 3, 2026 letter to Genevieve Bissonette) and Government of Alberta responses to municipal council motions calling for the transition to be paused.
5. *Convention on the Rights of Persons with Disabilities*, Article 4(3); Canada signed March 30, 2007, ratified March 11, 2010; Committee on the Rights of Persons with Disabilities, General Comment No. 7 (2018) on the participation of persons with disabilities through their representative organizations, CRPD/C/GC/7. (Discussed at length in Chapter II.)
6. *Premier’s Council on the Status of Persons with Disabilities Act*, RSA 2000, c P-21; Government of Alberta, Premier’s Council on the Status of Persons with Disabilities, Annual Report 2022–2023 (documenting the subcommittee structure, including the Indigenous people with disabilities subcommittee, and the 2023–2026 Strategic Plan survey methodology and response total).
7. Government of Alberta, Premier’s Council Annual Report 2022–2023 (381 Strategic Plan survey responses); The Alberta Disability System Breakdown, *Consultation Gap Rebuttal*, April 2026, Layer 5 (the Premier’s Council bypass).
8. The Alberta Disability System Breakdown, *Consultation Gap Rebuttal*, April 2026, Layer 2 (the undisaggregated participant claim) and Layer 4 (the petition as documented empirical rebuttal).
9. Government of Alberta, *Transforming Disability Income Assistance in Alberta: Discussion Guide*, August 13, 2025; Government of Alberta, ADAP engagement page (consultation window August 13 – September 12, 2025); Inclusion Alberta, *ADAP Fact Sheet*, October and December 2025 (documenting the revisions to employment income exemption figures and the appeal-rights provisions that emerged after consultation closed). Bill 12 introduced November 25, 2025.
10. Darryl Learie, Petition Against the Provincial Government’s New Alberta Disability Assistance Program (ADAP), October 8, 2025, polioalberta.ca; tabled at the Alberta Legislature with documented signatures.
11. Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; MS Canada, “MS Canada calls to protect disability supports in Alberta,” November 28, 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Disability Action Hall, 2025 record at actionhall.ca/2025/; Bev Muendel-Atherstone (Friends of Medicare Lethbridge), Lethbridge Herald, October 2025. Municipal coalition documented in The Alberta Disability System Breakdown Coalition Ledger, May 2026.
12. The Alberta Disability System Breakdown, *The Engagement Page Record: A Forensic Timeline*, May 2026.
- 12a. Global News, “Alberta Premier Danielle Smith shuffles cabinet,” May 21, 2026, globalnews.ca/news/11858865/; CBC News coverage of the May 21, 2026 cabinet shuffle. Resignations of Finance Minister Nate Horner and

Hospitals Minister Matt Jones; Jason Nixon to Finance and President of Treasury Board; Nathan Neudorf (Lethbridge-East) to Assisted Living and Social Services. Coincidence with the AISH-to-ADAP transition timeline is documentary, not attributed: the ministerial reassignment falls between the May 12 regulation filing and the July 1 transition launch.

13. United Nations Committee on the Rights of Persons with Disabilities, *Concluding Observations on the combined second and third periodic reports of Canada*, UN Doc CRPD/C/CAN/CO/2-3 (Geneva: UN, 15 April 2025); adopted at the Committee's 779th meeting on 18 March 2025, during its 32nd session (3–21 March 2025), digitallibrary.un.org/record/4080760.
14. CRPD Committee Concluding Observations on Canada, March 18, 2025, recommendation on legally defined federal mechanisms and the conditioning of fiscal transfers on provincial CRPD adherence.
15. Alberta Ombudsman, Appeals Secretariat information page (public-facing description of pre-Bill-12 appeal route); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section on the pre-Bill-12 Citizens Appeal Panel.
16. Bill 12, sections governing the Medical Review Panel and the abolition of the Citizens Appeal Panel; Inclusion Alberta, *ADAP Fact Sheet*, December 2025; Zachary Weeks, "Bill 12 and ADAP," November 28, 2025.
17. Bill 12, section 12.8; CBC News (Bellefontaine), August 21, 2025 reporting on the ADAP Discussion Guide.
18. The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section comparing pre-Bill-12 and post-Bill-12 appeal rights structures.
19. See Chapter I and its associated endnotes for the full record of the 1998 Bill 26 episode (the *Institutional Confinement and Sexual Sterilization Compensation Act* of 1998), distinct in subject and provenance from the 2018 NDP Bill 26 (*An Act to Combat Poverty and Fight for Albertans with Disabilities*).
20. See *Convention on the Rights of Persons with Disabilities*, Articles 4(3), 12, 13, 19, 21, and 28; The Alberta Disability System Breakdown, *Charter and CRPD Analysis*, April 2026, mapping the articles to the documented operational practices under Bill 12.

Chapter V

The Information Asymmetry Condition

The Right to Know Before Consent

The Upstream Condition

Chapter IV closed on a forward reference. The chapter that preceded examined whether the consultation undertaken in connection with Bill 12 met the standard CRPD Article 4(3) requires. The documented record established that it did not. That chapter closed by naming the upstream condition without which the consultation standard cannot operate: the right of the affected population to know, in advance, what they are being asked to consent to.

This chapter examines that condition. The claim it advances is narrow and structural. Consent presupposes information. A population that is not in a position to know the operational rules of the program being designed for them, the criteria against which their continued eligibility will be assessed, or the standards by which the documentation they are required to produce will be evaluated, cannot meaningfully consent to the program. The consultation standard CRPD Article 4(3) requires is operationally hollow where the upstream information condition is not met. The hollow standard is the standard the Government of Alberta operated under in 2025. The chapter that follows documents how.

The Standards That Apply

Two articles of the Convention on the Rights of Persons with Disabilities are directly engaged. Article 9, the accessibility provision, requires State Parties to take appropriate measures to ensure that persons with disabilities have access, on an equal basis with others, to information and communications, including information and communications technologies and systems. Article 21, the freedom of expression and access to information provision, requires State Parties to ensure that information intended for the general public is provided in accessible formats and technologies appropriate to different kinds of disabilities, in a timely manner and without additional cost.¹

The domestic framework adds further specificity. The federal *Accessible Canada Act*, SC 2019, c 10, enacted as the domestic operationalization of Canada's CRPD obligations in the federal sphere, establishes the statutory goal of a Canada without barriers by January 1, 2040. The Act binds federally regulated organizations and federally delivered programs. The federal components implicated in the disability-support architecture — the Canada Revenue Agency, Service Canada, the Disability Tax Credit, and the Canada Disability Benefit — are within scope.²

At the provincial level, the *Alberta Human Rights Act* prohibits discrimination on the basis of physical and mental disability in the provision of government services. Section 4 establishes an affirmative duty of accommodation triggered when the service provider is on notice that a barrier exists — not when the affected individual files a formal request. At the constitutional level, Section 7 of the Canadian Charter of Rights and Freedoms protects against deprivation of life, liberty, or security on the basis of evidence the affected person cannot review, criteria they cannot evaluate, or processes in which they cannot meaningfully participate. The Supreme Court of Canada in *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624, established that Section 15 of the Charter requires substantive equality — not merely identical text on paper, but the actual capacity of protected groups to exercise their rights on equal terms.³

Plain-language standards operationalize what accessibility means for written communications. Federal plain-language guidance, including the Plain Language Action and Information Network (PLAIN) standards and the Treasury Board Secretariat’s communications guidance, has historically recommended a Grade 8 reading level or below for general public communications. The disability-advocacy community recommends Grade 6 or below for communications directed at populations with cognitive or intellectual disabilities. The federal Plain Language Action and Information Network defines plain language as writing that the intended audience can read, understand, and act on the first time they read it. Statistics Canada’s second cycle of the Programme for the International Assessment of Adult Competencies (PIAAC), with data collected in 2022 and 2023 and released December 10, 2024, places approximately forty-nine percent of Canadian adults below the Level 3 threshold required for everyday government and health communications, with nineteen percent at Level 1 or below.⁴

The Reading-Level Gap

Independent readability analysis of Government of Alberta AISH program documentation, ADAP transition information, and federal Canada Disability Benefit application materials places the typical reading level between Grade 12 and Grade 16 — that is, university-undergraduate to graduate-level English. A document at this reading level is comprehensible to approximately twelve percent of Canadian adults at first-read full comprehension.⁵

The reading level matters because of who is in the AISH caseload. The Government of Alberta’s own caseload data as of September 2025 identifies primary medical conditions across the 79,290 recipients as follows: 22,419 (28.3 percent) with cognitive disorders as their primary diagnosis, including autism, intellectual disability, acquired brain injury, and severe dementia; 23,940 (30.2 percent) with mental illness as their primary diagnosis, including major depressive disorder, severe anxiety, schizophrenia, and post-traumatic stress disorder; approximately 24,800 (31.3 percent) with physical disability, including mobility, sensory, and chronic-illness conditions; and approximately 8,131 (10.2 percent) with other or multiple primary diagnoses. Combined, more than 58 percent of the AISH caseload — 46,359 people — have a primary diagnosis of cognitive disorder or mental illness.⁶

The defining clinical features of these conditions include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step

administrative tasks, difficulty navigating institutional systems without external support, and difficulty maintaining sustained attention across multi-page documents. These are not aspersions. They are the diagnostic criteria. They are the basis on which the Government of Alberta approved each recipient as eligible for AISH in the first place.

The same Government of Alberta then designed a compliance system requiring those same recipients to read forms written at Grade 12 to 16 reading level, to complete multi-step paperwork chains across multiple agencies, to find a family doctor in a province where approximately 650,000 people have no family physician, to pay physician fees out of pocket on an income below the poverty line, and to navigate phone trees, online portals, and appeal processes without integrated accommodation. The defining clinical features of the population the program was designed to serve are the defining features the program then requires the population to override in order to remain on it.⁷

What Was Not Published Before the Transition

The information that the Government of Alberta did not publish, in any form that a prospective ADAP recipient could review before the consultation closed in September 2025, is the category of information most directly relevant to the consent question Chapter IV examined. The list is documented in the campaign's *AISH Evidentiary Report Series* and the *Compound Failure Brief*; the items below are reproduced with the campaign's primary-source verification.⁸

The basis for the reversal of prior findings. Every AISH recipient was assessed and approved by the Government of Alberta's formal adjudication process. The ADAP transition reverses those findings for all 79,290 recipients simultaneously. The Government has not published, in any form, the medical, administrative, or legal basis on which the prior findings are treated as insufficient to maintain eligibility without reapplication. The reversal is operative; the justification is undocumented.

The criteria distinguishing AISH from ADAP eligibility. Minister Jason Nixon confirmed in correspondence dated April 3, 2026 that the employment income exemption calculations under the post-transition framework would be set in a Ministerial Order "later this spring." The transition is effective July 1, 2026. The operational rules governing the income reduction structure that will apply to 79,290 people were not published when those people were consulted in the engagement window that closed in September 2025. They were not published when Bill 12 was passed in December 2025. They were not published as the campaign's most recent documentation of the matter went to publication in May 2026.⁹

The May 12, 2026 regulations and the operational confirmation of the asymmetry. The Government of Alberta filed the ADAP regulations on May 12, 2026 (Alberta Regulation 96/2026), published through the *Alberta Gazette* the same week. The regulations set the dollar amounts of the post-transition benefits (\$1,940 monthly for AISH; \$1,740 monthly for ADAP), codified the cohabitation reduction at 88 percent for two disability-benefit recipients living together, and modified the child-benefit structure (increase to \$300 monthly for the first child; reduction for families with four or more children). The regulations did not define the substantive criteria that adjudicators will apply to distinguish AISH eligibility ("severe disability that permanently prevents employment") from ADAP eligibility ("disability that substantially

impedes employment”). The Ministerial Order Nixon had referenced was not the document filed. As of the date of this edition, May 23, 2026, five weeks before the July 1, 2026 transition, the criteria governing whether a particular recipient remains on AISH or moves to ADAP have not been published.^{9a}

The May 2026 individualized eligibility notification. In the same week the regulations were filed without the eligibility criteria, the Government of Alberta began mailing individualized eligibility-determination letters to the entire AISH caseload. Each of the approximately 80,000 current AISH recipients is receiving a letter informing them of one of three outcomes: they remain on AISH; they transition to ADAP; or their file is otherwise affected by the regulatory change. The Government’s public projection is that approximately 50,000 recipients will be moved to ADAP and approximately 30,000 will remain on AISH. The individualized determinations are being made — and communicated to recipients — on the basis of criteria the Government has not published. Recipients are being informed of the outcome of a determination they cannot interrogate, on the basis of a standard they cannot read, in a process for which the right of appeal was removed by Bill 12 in December 2025. The Information Asymmetry condition this chapter documents is, in the week of this edition, operationally complete. The Government holds the criteria; the population the criteria are being applied to does not.^{9b}

The Medical Review Panel adjudication standards. The Panel that replaces the Citizens Appeal Panel as the final decision-maker on AISH eligibility under the post-transition framework will apply criteria the Government has not published. The specific criteria the medical adjudicator applies to determine “substantial limitation of ability to earn a livelihood” have not been disclosed. The evidentiary thresholds for “permanence” of a condition have not been disclosed. The assessment standards for fluctuating conditions — which do not present consistently across any single assessment period and which characterize many of the mental health and chronic-pain conditions in the caseload — have not been disclosed. The criteria for determining “likely improvement,” a determination that requires the adjudicator to make a prognosis the applicant’s own physicians may not have made, have not been disclosed.¹⁰

The structural “framing-amenable” criterion. The campaign’s *Compound Failure Brief* identifies an operative criterion that the Medical Review Panel will, in functional effect, apply: not whether a recipient’s disability is real or severe (the existing file already establishes that, and the Government has accepted the equivalent established record for every recipient in an exempted category) but whether a recipient can be plausibly framed as a candidate for employment. The exemption categories themselves identify the criterion: recipients over sixty are exempted because their employment trajectory is no longer considered relevant; PDD recipients are exempted because their developmental disability is considered conclusive of unemployability; continuing-care residents are exempted because their living situation is conclusive; palliative recipients are exempted because their prognosis is conclusive. The transitioning cohort is the cohort the Government considers framing-amenable. The criterion is not in any published document. Its operation is documented in the structure of the exemptions.¹¹

The recipient’s position. The applicant whose file is being newly evaluated under this unpublished criterion has no opportunity to review what is being applied to them, no opportunity to respond to it, and no independent body to which an erroneous application can be appealed. The criterion is being applied by appointees of the ministry whose budget benefits from each ADAP placement, in the absence of the

appeal panel that historically reviewed decisions of this kind. The Government's existing record on the recipient — the record establishing that the same Government previously found this person unable to sustain a livelihood — is held in the same ministry's filing cabinet as the new determination is being made.

The Compounding Effect Across Programs

The information-asymmetry pattern is not unique to AISH and ADAP. It compounds across the federal-provincial disability-support chain. The federal Canada Disability Benefit requires prior approval for the Disability Tax Credit. The DTC requires a physician to complete the T2201 form. Physician fees for T2201 completion are unregulated and range from approximately fifty to three hundred dollars or more across the province. The Government of Canada's 2024 Disability Advisory Committee report documents a twenty-four percent completion rate for online DTC applications — equivalently, a seventy-six percent abandonment rate. The Disability Advisory Committee identifies administrative complexity as a documented access barrier rather than a measure of ineligibility.¹²

The same population that must complete the DTC to qualify for the CDB is the population whose CDB Alberta then claws back dollar-for-dollar from the provincial AISH or ADAP cheque. The documentation chain is documented in the campaign's *Access Barrier Report* and traced through timestamped correspondence in a primary-source case file spanning 2019 to 2026. The chain's practical effect is that a recipient who succeeds in completing all of the federal access requirements receives zero net financial benefit, and a recipient who fails in completing them sees their AISH cheque reduced by two hundred dollars per month regardless.¹³

The same pattern operates in the child and developmental-disability programs. The Family Support for Children with Disabilities (FSCD) and Persons with Developmental Disabilities (PDD) program forms inherit the same readability and accessibility issues with one structural addition: the parent or guardian completing the form is often themselves managing the disability of the child or family member requiring the support. The administrative complexity does not adjust for caregiver load. The bureaucratic shorthand does not soften when the household is in crisis. Families drop out of programs they were eligible for, or never apply at all.¹⁴

The Structural Contradiction

The integrated effect of the documented pattern is a structural contradiction that can be stated precisely. The Government of Alberta determined, through its own formal adjudication process, that 79,290 people have severe and permanent disabilities preventing them from earning a livelihood. The defining clinical features of the largest groups in that population — cognitive disorders, mental illness — include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, and difficulty navigating institutional systems without external support. The same Government then designed the compliance, consultation, and reapplication architecture under which the same population is required to read forms at Grade 12 to 16 reading level, navigate multi-step bureaucratic processes across multiple agencies, meet deadlines without flexibility for medical flares or hospitalizations, and respond to consultation invitations whose subject matter is the criteria that will subsequently be applied to determine their continued eligibility — criteria the Government has not published.¹⁵

The contradiction is not incidental. It is the operating structure of the program. The recipient population was assessed and approved into AISH because of the limitations that make the compliance system inaccessible. The same limitations are then operationally treated, by the program's communications and processes, as if they were not present. A population whose qualifying conditions include difficulty with multi-step administrative tasks is required to perform multi-step administrative tasks in order to keep the support that recognized the difficulty in the first place.

The community response to the documented pattern is itself a data point. The Alberta AISH recipient community group tripled in size over the period from spring 2025 to spring 2026, from approximately two thousand members to more than six thousand, according to the group's administrator. Community groups of this kind do not triple in size when recipients feel supported and informed. They triple when recipients feel abandoned and afraid, and when they are actively seeking information and community that the formal system has ceased to provide. The growth of informal recipient-led networks during the same period the formal supports were being removed is not coincidence. It is the predictable consequence of the removal.¹⁶

The Constructive Knowledge Doctrine

A principle in administrative law and in human rights jurisprudence operates across the situation this chapter documents. Where an institution knows, or where in the circumstances it ought to know, that a standard it is applying cannot be met by the population to whom it is applied, the institution's continued application of the standard is not procedurally innocent. The constructive-knowledge doctrine attaches the institution's knowledge to its conduct regardless of whether each individual decision-maker subjectively intends the foreseeable result.¹⁷

The Government of Alberta is on documented constructive notice of every element examined in this chapter. The 79,290 caseload figure and the breakdown by primary medical condition are from its own open data. The Grade 12 to 16 reading level of its forms is a function of its own document drafting practice. The non-publication of the Medical Review Panel adjudication standards is its own

administrative choice. The Ministerial Order setting the employment income exemption calculation is within its own authority to issue or withhold. The 24 percent DTC completion rate is documented in federal reports the Government has the operational capacity to consult. The reading-level standards in federal Treasury Board guidance, in disability-advocacy practice, and in the Convention itself are public and accessible.

The Government has been formally notified of the pattern documented here through Inclusion Alberta's public analyses, through the campaign's evidentiary reports published and distributed to government addressees, through municipal council motions, through the November 26, 2025 Inclusion Alberta media release, through the MS Canada statement of November 28, 2025, through Alberta Medical Association publications including the November/December 2025 issue of *Alberta Doctors' Digest*, and through direct ministerial correspondence from named individual recipients. The constructive knowledge condition is met as a matter of documented record.¹⁸

Where the constructive knowledge condition is met, the continued operation of the inaccessible standard is not an administrative failure subject to incremental reform. It is the operating design of the program. The information-asymmetry condition is, on the documented record, sustained because it produces the outcomes the surrounding fiscal architecture serves. The recipient who cannot complete the reapplication does not remain on AISH. The applicant who cannot navigate the DTC chain does not access the CDB. The family that cannot parse the FSCD documentation does not receive the support. The caseload reduces by attrition. The fiscal saving is the saving the policy framework was structured to produce.

What This Chapter Establishes

Four observations follow.

First, the CRPD Article 4(3) consultation standard examined in Chapter IV cannot meaningfully operate where the upstream information condition is not met. A population that has not been informed of the operational rules of the program being designed for them, of the criteria against which their continued eligibility will be assessed, or of the standards by which their documentation will be evaluated, cannot meaningfully consent to the program through any consultation mechanism the consulting authority designs.

Second, the documented information condition in Alberta as of the consultation window in September 2025 and as of the transition window in July 2026 fails the standards CRPD Articles 9 and 21 establish, the standards the federal *Accessible Canada Act* articulates, and the standards the *Alberta Human Rights Act* Section 4 duty of accommodation triggers. The failure is not incidental. It is documented in the reading-level of the forms, in the non-publication of the adjudication standards, in the deferral of the Ministerial Order, and in the structural contradiction between the qualifying conditions and the navigational demands.

Third, the structural contradiction is precise. The population the program was designed to serve was assessed and approved because of the limitations that make the program's communications and processes inaccessible. The same limitations are then operationally treated as if they were not present. The

diagnostic criteria on which approval was granted are the diagnostic criteria the maintenance system then requires the recipient to override.

Fourth, the constructive knowledge doctrine is engaged on the documented record. The Government of Alberta is on notice, through its own open data, through federal accessibility jurisprudence, through formal correspondence from civil society organizations, and through municipal advocacy, of every element of the pattern this chapter documents. The continued operation of the inaccessible architecture, in the face of the documented pattern, is not an administrative failure capable of incremental reform without a change in the institutional position. It is the operating structure that produces the fiscal outcomes the surrounding policy framework was designed to produce.

The chapter that follows examines what is being built in the institutional space the consultation standard would have constrained — the architecture currently under construction in Alberta and in jurisdictions adjacent to it. The information condition documented in this chapter is the condition under which that construction has been authorized to proceed.

Notes — Chapter V

1. *Convention on the Rights of Persons with Disabilities*, Articles 9 (Accessibility) and 21 (Freedom of Expression and Access to Information); Canada ratified March 11, 2010. The full text of both articles is available at the UN Department of Economic and Social Affairs, Division for Inclusive Social Development, <https://www.un.org/development/desa/disabilities/>.
2. *Accessible Canada Act*, SC 2019, c 10; Bill C-81, *An Act to Ensure a Barrier-Free Canada*, tabled June 20, 2018 by the Honourable Carla Qualtrough, then Minister of Public Services and Procurement and Accessibility; Royal Assent June 21, 2019; in force July 11, 2019. Statutory objective: realization of a Canada without barriers by January 1, 2040.
3. *Alberta Human Rights Act*, RSA 2000, c A-25.5, section 4; *Canadian Charter of Rights and Freedoms*, sections 7 and 15, Part I of the *Constitution Act, 1982*; *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624 (substantive equality requires accommodation rather than merely identical text on paper).
4. Government of Canada, Treasury Board Secretariat, *Policy on Communications and Federal Identity* (effective March 27, 2025) and *Canada.ca Content Style Guide*; Plain Language Action and Information Network (PLAIN), plainlanguage.gov; Statistics Canada, *Literacy, numeracy and adaptive problem-solving skills of Canadians: Results from the 2022 Programme for the International Assessment of Adult Competencies*, released December 10, 2024, www150.statcan.gc.ca/n1/daily-quotidien/241210/dq241210a-eng.htm (Cycle 2: 19 percent of Canadian adults at literacy Level 1 or below; approximately 49 percent below Level 3).
5. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5 (Bureaucratic Legalese as Gatekeeping). Independent readability analysis using standard tools (Flesch-Kincaid Grade Level) of Government of Alberta AISH and ADAP documentation and federal CDB application materials.
6. Government of Alberta, AISH Caseload Open Data, September 2025, open.alberta.ca; The Alberta Disability System Breakdown, *What Disability Actually Means*, May 2026 (plain-language guide documenting the caseload breakdown by primary medical condition).
7. Government of Alberta, family physician availability data; The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 1 (The Structural Contradiction).
8. The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, Section 6 (The Questions That Require Answers Before July 1, 2026); *Compound Failure Brief*, April 2026, Pillar III(d) (the Removal of Appeal Rights and the Medical Review Panel architecture).
9. Minister Jason Nixon, Minister of Assisted Living and Social Services, letter to Genevieve Bissonette, April 3, 2026 (confirming the Ministerial Order on employment income calculations would be issued “later this spring”); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, on the deferred Ministerial Order.
- 9a. Alberta Regulation 96/2026 (the ADAP regulation), filed May 12, 2026, in force July 2, 2026. Coverage: Medicine Hat News, “More rules, reductions announced for ADAP/AISH clients,” May 14, 2026, medicinehatnews.com. The regulation sets the dollar amounts of the post-transition benefits, the cohabitation reduction at 88 percent, and the modified child-benefit structure. The eligibility criteria distinguishing AISH (“permanently prevents employment”) from ADAP (“substantially impedes employment”) are not set out in the regulation and have not been published as a Ministerial Order through the *Alberta Gazette* as of the date of this edition.
- 9b. Red Deer Advocate, “Disabled Albertans to receive letters this month about their AISH eligibility,” May 13, 2026, reddeeradvocate.com. The Government’s projection of approximately 50,000 ADAP recipients and 30,000 remaining on AISH is the public statement of Minister Jason Nixon at the regulation filing. Inclusion Alberta has reported “disappointment, fear and confusion” among individuals and families contacting the organization. The removal of the right of appeal by Bill 12 (*Financial Statutes Amendment Act, 2025 (No. 2)*, SA 2025, c 20, section amending the AISH Act’s appeal provisions) is documented in Chapter IV.

10. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), documenting the specific categories of adjudication criteria that have not been published in any form a prospective applicant could review before the post-transition Medical Review Panel adjudication.
11. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), analysis of the exemption-category structure and the operative “framing-amenable” criterion derived from it.
12. Government of Canada Disability Advisory Committee, 2024 report; documented 24 percent completion rate for online DTC applications and 76 percent abandonment rate. DTC physician fee structure documented in The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, with primary-source receipts and correspondence.
13. The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, documenting the federal-provincial DTC/CDB/AISH chain through timestamped correspondence in a primary-source case file spanning 2019 to 2026.
14. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5.3 (The FSCD/PDD Form Comparison). The pattern of caregiver-mediated administrative complexity inheriting the same readability and accessibility issues as the recipient-mediated AISH/ADAP architecture.
15. See note 7 and accompanying text; The Alberta Disability System Breakdown, *Information Asymmetry Plain Language Companion*, April 2026, naming the structural contradiction at the policy-design level.
16. Community group growth documented in the campaign’s observations of the Alberta AISH recipient community group — the same kind of network that emerged in disability communities historically when formal support structures failed; see The Alberta Disability System Breakdown, *Indigenous Disability Alberta Collection*, May 2026, for the parallel pattern in Indigenous disability communities during the same period.
17. The constructive-knowledge doctrine operates across multiple branches of Canadian and international law, including administrative-law jurisprudence on procedural fairness, human-rights jurisprudence on the duty to accommodate (see, e.g., *British Columbia (Public Service Employee Relations Commission) v. BCGSEU*, [1999] 3 SCR 3 (the *Meiorin* standard)), and tort jurisprudence on knowing or willful continuation of harmful conduct. The doctrine’s application to the disability-policy context is developed in The Alberta Disability System Breakdown, *Charter and CRPD Analysis*, April 2026.
18. Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; MS Canada, November 28, 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; The Alberta Disability System Breakdown distribution record covering Tier 1 (provincial, federal, oversight, municipal), Tier 2 (coalition), and Tier 3 (media) recipients, 2025–2026.

Chapter VI

Modern Eugenics Momentum

*The Institutional Rebuild***What Is Being Built**

Across the period 2024 to 2026, a cluster of legislative and executive measures has expanded the legal authority of governments in Canada and the United States to detain and to compel treatment of persons with severe substance-use, mental health, and concurrent disorders. The measures are not identical. Each is responding to a different combination of opioid mortality, encampment crisis, and political pressure. The legal frameworks they create differ in their procedural safeguards, their authorizing officials, and their scope. The pattern, taken together, is consistent: legislative authority to compel treatment is expanding rapidly in jurisdictions that are, in the same period, constraining the community-based supports through which voluntary care has historically been delivered.

This chapter documents what has been built. Five jurisdictions are examined: Alberta, Saskatchewan, British Columbia, Ontario, and the United States at the federal level. The chapter then applies the framework Chapter II established — the structural-eugenics analysis — to the documented pattern. The structural argument is the effect argument. It is not the claim that contemporary governments harbour the explicit eugenic intent of the early twentieth century. It is the claim that the pattern of building large secure-treatment infrastructure for predominantly poor, disabled, and disproportionately Indigenous populations, in the absence of equivalent investment in community-based alternatives, is structurally continuous with the institutionalization pattern documented in Chapter I — and that the documented overlap of the affected populations with the populations historically subject to the *Sexual Sterilization Act* is not incidental.

Alberta: The Compassionate Intervention Act

On April 15, 2025, the Government of Alberta introduced Bill 53, the *Compassionate Intervention Act*. The Act received Royal Assent on May 15, 2025. It establishes a framework for involuntary treatment of persons with severe substance-use or addiction whose conduct is determined to be likely to cause substantial harm to themselves or others within a reasonable time.¹

The Act's operative provisions authorize adult family members, guardians, healthcare professionals, police officers, and peace officers to apply for an assessment order. A three-person Compassionate Intervention Commission, composed of a lawyer, a physician, and a member of the public, all appointed by the provincial government, reviews applications and issues legally binding treatment decisions. The Act provides for a 72-hour assessment and detoxification period during which a person may be detained

for assessment, with hearings before the full Commission required within that window. Secure treatment orders of up to three months may be issued; community-based treatment orders may extend to six months. Care-plan reviews occur every three weeks during the treatment period. The Act replaces the 2006 *Protection of Children Abusing Drugs Act* (PChAD) for youth.²

The Act expands the provincial involuntary-treatment framework beyond the existing *Mental Health Act*, RSA 2000, c M-13. The *Mental Health Act* framework operates against persons with mental disorders meeting an impaired-capacity test. The *Compassionate Intervention Act* operates against persons with severe substance-use or addiction even where the capacity test is met. The substantive evidentiary threshold for involuntary detention and treatment under the new framework is lower than the threshold the existing *Mental Health Act* infrastructure required.³

The infrastructure investment associated with the Act has been progressively announced. Budget 2025 allocated \$180 million; Budget 2026 increased the total allocation to \$318.6 million over three years (2026–2029), of which \$291 million funds two 150-bed compassionate-intervention centres in Edmonton and Calgary, expected to open in 2030. Operating funding accompanies the build: approximately \$33 million for compassionate intervention through Recovery Alberta, which administers the framework, and a \$59 million increase to hospital and continuing-care services supporting the rollout. Approximately 100 secure beds are being established in existing facilities during the implementation phase.⁴

The professional-and-academic response has been substantial. Eric Adams, a constitutional law professor at the University of Alberta, has stated that the legislation is expected to face Charter challenge once it becomes operational, given its interference with Charter Section 7 rights (life, liberty, security of the person) and Section 9 rights (freedom from arbitrary detention). The Canadian Bar Association — Alberta Branch, in a May 9, 2025 letter, raised concerns including the substantive overlap with existing legislation (the *Personal Directives Act*, the *Adult Guardianship and Trusteeship Act*, the *Youth Criminal Justice Act*), procedural-fairness questions including the ability of the “client” to participate in the assessment process, presumption-of-capacity questions, and consent-and-treatment provisions including authorization of medication administration without consent.⁵

The *Canadian Medical Association Journal* published an ethical analysis applying the Kass framework for public-health interventions to the Act, concluding that the framework presents substantive risks across all three Kass-framework domains — privacy, liberty, and justice — with operational consequences including the potential for disproportionate targeting of structurally marginalized populations including First Nations, Inuit, and Métis individuals. The Alberta Medical Association, in a May 7, 2025 statement, identified operational concerns including post-discharge care, legal safeguards, and the operational interaction with social factors including housing and income. Field Law, a regulatory law firm advising health professionals, published a January 6, 2026 analysis documenting operational tensions between the new framework and the existing *Mental Health Act* infrastructure.⁶

The opioid-crisis context within which the Act has been developed is severe. Alberta documented more than 1,800 opioid-related deaths in 2023 and 1,189 deaths in 2024. The March 2025 Edmonton-zone single-month record reached 87 deaths. Carfentanil involvement reached 68 percent of provincial overdoses and 78 percent of Edmonton overdoses in the January to May 2025 period. Opioid Dependency Program enrollment grew from 3,288 clients in 2018 to 12,299 clients in 2024. The provincial policy

direction during the same period has shifted from harm reduction toward recovery-focused approaches, with supervised-consumption-site closures across the province.⁷

Saskatchewan: A Parallel Statute

On December 5, 2025, Premier Scott Moe’s government in Saskatchewan introduced legislation under the same name as Alberta’s: the *Compassionate Intervention Act*. The Saskatchewan bill was tabled on the final day of the fall legislative sitting. The legislation passed on May 5, 2026. The Act is expected to come into force in fall 2026 after regulations are finalized.⁸

The Saskatchewan framework follows the Alberta architecture in substantive structure. Family members may request treatment through the court system; law enforcement may intervene where substance use puts life at serious risk; medical professionals may refer. The first Compassionate Intervention Assessment Centre will be located in North Battleford. The involuntary inpatient unit will be situated at Saskatchewan Hospital North Battleford, an existing psychiatric facility. The Government of Saskatchewan reports that approximately 150 stakeholders participated in information sessions during the legislation’s development.⁹

The opposition record across the Saskatchewan disability, medical, and civil-rights sector mirrors the Alberta record. The Saskatchewan Medical Association issued a joint statement with other physician organizations identifying capacity concerns: “The healthcare system is not ready for this. Emergency departments and other frontline services are already under severe strain, and the system does not have the capacity required to safely implement involuntary pathways.” The John Howard Society of Saskatchewan, which advocates for incarcerated people, echoed the physicians’ concerns. The Opposition NDP voted against the bill, with leader Carla Beck arguing the province had not put adequate protections in place.¹⁰

The Government cited support from Saskatoon Tribal Chief Mark Arcand, who noted the framework’s stated focus on culturally responsive care. The endorsement is, on the documented record, one of relatively few from Indigenous leadership; the broader Indigenous-disability and First-Nations health-policy sector has not, in the documented public record, endorsed the framework.¹¹

British Columbia: Mental Health Act Expansion

British Columbia has expanded involuntary care under its existing *Mental Health Act* framework rather than through a separate Compassionate Intervention Act. On September 15, 2024, Premier David Eby announced a secure-care framework targeting people with concurrent addiction, mental illness, and acquired brain injuries from toxic-drug poisonings. The provincial framework, on the documented public record, is the most direct response to a provincial overdose crisis that has produced more than 15,000 deaths since 2016, when the province declared a public health emergency.¹²

In summer 2024, the province appointed Dr. Daniel Vigo as British Columbia’s first chief scientific adviser for psychiatry, toxic drugs and concurrent disorders. On March 12, 2025, Dr. Vigo issued clinical guidance specifying that involuntary care under the *Mental Health Act* cannot be used to prevent harmful “risk-taking” by people who use drugs whose behaviour is not related to mental impairment. The guidance

attempts to articulate the boundary between the legitimate medical application of involuntary care for persons whose decision-making capacity is impaired by severe mental illness and the use of involuntary care as a policy response to public-space disorder. The boundary is, in the documented operational practice that has followed, subject to substantive interpretive variation across health authorities and facilities.¹³

The first correctional involuntary-care facility, a 10-bed unit at the Surrey Pretrial Services Centre, opened on April 24, 2025. The first secure housing-and-care facility, Alouette Homes on the grounds of the Alouette Correctional Centre in Maple Ridge, opened in June 2025 with 18 long-term involuntary care beds. More than 400 mental-health beds are being built or modernized across the province — approximately 280 outdated beds modernized and more than 140 new beds added — under the framework. On November 24, 2025, the province introduced *Mental Health Act* amendments operationalizing the framework expansion.¹⁴

The framework targets, in the language of provincial communications, people with “concurrent mental-health and substance-use challenges, and brain injuries from toxic-drug poisonings.” The triple-condition profile the framework engages overlaps substantively with the documentation in the British Columbia Centre on Substance Use literature on the population most affected by the toxic drug supply — a population the same Centre has identified as inadequately served by the involuntary-care expansion in the absence of comparable expansion in voluntary community-based services.¹⁵

Ontario: The Encampment Frame

Ontario has not, as of the date of this chapter, passed compassionate-intervention legislation parallel to Alberta’s or Saskatchewan’s. The Ontario framework is moving more slowly, but along a recognizable trajectory. In May 2025, the province announced a study of involuntary addictions treatment for people in the correctional system — in jail, on probation, on parole. The Ontario approach explicitly cites the British Columbia model.¹⁶

The Ontario opioid context is, in absolute terms, the largest in Canada. The province documented 2,600 or more opioid-related deaths in 2023, a 50 percent increase over 2019. Approximately 48 percent of inmates carry substance-use alerts on their profiles (February 2024 data); approximately 28 percent of inmates are voluntarily on prescribed opioid agonist therapy. The Canadian Mental Health Association — Ontario has identified 1,400 or more encampments across the province and an average of seven deaths per day from drug poisonings, with \$18 billion in 2020 social costs from substance use.¹⁷

The political pressure for expansion has come substantively from municipal leadership. In October 2024, Ontario Big City Mayors — an organization representing 29 mayors of cities over 100,000 population — collectively asked the province to review mental-health laws and the scope of involuntary treatment. Mayor Alex Nuttall of Barrie and Mayor Cam Guthrie of Guelph have been particularly outspoken. The CMHA Ontario position, by contrast, has been to oppose involuntary-treatment expansion and to ask for \$113 million in the 2025-26 budget for community mental health, including 5,000 additional supportive housing units. The expansion of involuntary treatment infrastructure and the expansion of community supportive-housing infrastructure are being requested by different actors at the same time. The province

has, on the documented record, moved more quickly on the first than on the second.¹⁸

Ontario does have an existing Bill 53, the *Dignity and Mental Health in Jails Act, 2025*, distinct from Alberta's Bill 53. The Ontario Bill 53 addresses correctional mental-health units, requires that 20 percent of inmate beds in correctional institutions be in mental-health support units, and prohibits cruel, inhumane, or degrading treatment within corrections. It is not, on its face, an involuntary-treatment expansion in the sense documented in Alberta and Saskatchewan, but it operates in the same conceptual space — the provision of mental-health treatment within carceral infrastructure.¹⁹

The United States: Executive Order 14321

On July 24, 2025, President Donald J. Trump signed Executive Order 14321, titled “Ending Crime and Disorder on America’s Streets.” The Order, which adopts pledges from the Project 2025 blueprint, expands the use of involuntary civil commitments for adults experiencing homelessness and serious mental illness, directs the Department of Justice to evaluate homeless individuals arrested for federal crimes as “sexually dangerous persons,” and ends federal funding for Housing First programs. It directs the Secretary of Health and Human Services and the Secretary of Housing and Urban Development to provide federal grants and legal assistance to state and local governments for “maximally flexible civil commitment, institutional treatment, and step-down treatment standards.”²⁰

The legal context is the 1999 United States Supreme Court decision *Olmstead v. L.C.*, 527 U.S. 581 — a twenty-six-year-settled precedent guaranteeing persons with disabilities the right to live in community with necessary support. The Executive Order, in the analysis of the National Alliance to End Homelessness, “defies a 26-year settled legal precedent in *Olmstead v. L.C.*” The 2024 Supreme Court decision in *Grants Pass v. Johnson*, which permits ticketing, fining, and arresting people sleeping on public property, provides the procedural environment within which the Executive Order’s institutionalization framework is being deployed.²¹

The opposition to the Executive Order has been broad. The National Alliance to End Homelessness, the Kaiser Family Foundation in an August 20, 2025 analysis, the law firm Epstein Becker Green in a July 30, 2025 legal review, the Prison Policy Initiative in an August 5, 2025 analysis, and Shannon Minter of the National Center for LGBTQ Rights have each identified substantive concerns. The Epstein Becker Green analysis observes that “the Executive Order does not include access to federal funds to build out treatment capacity for people who are civilly committed” and that “the Trump administration’s efforts so far have involved cutting funds for mental health treatment, not offering to fund increased capacity.”²²

The Hari/Rat Park Frame Reapplied

The framework Chapter II established turns on the distinction between policy responses to addiction that operate through isolation and policy responses that operate through connection. The Rat Park studies conducted by Bruce K. Alexander and colleagues at Simon Fraser University in the 1970s established experimentally what the public-health literature has subsequently confirmed: isolated, deprived environments produce predictable adaptive responses, including substance dependence, that are mispathologized as individual failure when the structural conditions producing them are not also examined. The work has been synthesized for general audiences most prominently by Johann Hari, whose central reframe is that the opposite of addiction is not sobriety but connection.²³

The Portugal decriminalization model, introduced in July 2001, is generally cited as the structural opposite of the mandate-treatment framework. Portugal's framework decriminalizes personal possession of small quantities of drugs and routes persons identified with problematic substance use into voluntary treatment, integrated with housing, employment, and community-reintegration supports. The documented outcomes, across European Monitoring Centre data and the academic literature, include substantial reductions in HIV transmission, overdose deaths, and the incarceration of drug users, alongside documented increases in treatment uptake.²⁴

The framework currently being built across Alberta, Saskatchewan, British Columbia, Ontario, and at the United States federal level operates on the opposite premise. The mechanism it deploys is mandatory treatment within secure facilities, often co-located with or adjacent to correctional infrastructure (Surrey Pretrial Services Centre; Alouette Correctional Centre grounds; the correctional-system pathway in Ontario; the Saskatchewan Hospital psychiatric facility), authorized under legal frameworks that lower the evidentiary threshold below the existing mental-health-act standards. The pattern occurs in the same jurisdictions and during the same period in which voluntary community-based supports are being constrained — through supervised-consumption-site closures, harm-reduction funding reductions, the reorientation of addictions services toward abstinence-only models, and (in the United States) the federal defunding of Housing First programs.²⁵

The structural argument is direct. A government that responds to a population's isolation by building larger isolation infrastructure, while simultaneously dismantling the community-based mechanisms through which that population's connection could be supported, has chosen the policy response the Rat Park literature identifies as the response that produces the pattern the policy is presented as addressing. The mechanism is recursive. The infrastructure expansion does not address the conditions producing the crisis; it expands the institutional capacity to manage the population the crisis has produced.

The Parallel to the Provincial Training School Period

Chapter I documented the institutional infrastructure of the Alberta eugenic period: the Provincial Training School at Red Deer, the Alberta Hospital network including Ponoka, Oliver, and Edmonton, the Michener Centre, and the auxiliary facilities through which persons identified as “feeble-minded,” “psychopathic,” or “mentally defective” were institutionalized and, in approximately 2,800 documented cases, sterilized under the authority of the Eugenics Board. The infrastructure was not built on explicit eugenic legislation alone. It was built on a broader consensus that institutional confinement was the appropriate response to populations the surrounding social and economic structure could not or would not accommodate.²⁶

The populations the historical institutionalization framework engaged are substantively the same populations the contemporary involuntary-treatment frameworks engage. The overlap is documented across multiple dimensions. The Alberta *Sexual Sterilization Act* period’s disproportionate impact on Indigenous women is paralleled by the CMAJ ethical analysis warning of disproportionate targeting of First Nations, Inuit, and Métis individuals under the *Compassionate Intervention Act*. The historical institutionalization of persons with cognitive disability and mental illness is paralleled by the contemporary framework’s targeting of persons with concurrent mental-health and substance-use conditions — conditions whose presentation frequently overlaps with cognitive disability and acquired brain injury. The historical institutionalization of poor persons whose poverty was characterized as moral failure is paralleled by the contemporary framework’s targeting of unhoused persons whose homelessness is characterized in the United States Executive Order as “crime and disorder.”²⁷

The mechanism of authorization differs. The Eugenics Board authorized sterilization. The contemporary Commissions and review boards authorize involuntary treatment and detention in secure facilities. Sterilization is not a contemporary operational tool of the framework. Detention and compelled treatment in secure facilities are.

The structural argument is the effect argument, not the intent argument. The framework that is being built does not require the eugenic intent of the 1928 legislation to produce structurally continuous outcomes for structurally continuous populations. It requires only the operating premise that some populations are properly subjects of institutional management rather than community participants — and the operational capacity to act on that premise at scale.

The Architecture Returns: Where the Frameworks Are Being Installed

The contemporary frameworks documented in this chapter are most often discussed in terms of their statutory provisions, their political sponsors, and their evidentiary justifications. Less often noted is where, physically, they are being installed. The question of physical location is not incidental. It is the evidentiary anchor of the institutional-continuity claim this piece develops.²⁸

Alberta: the retrofit list. Budget 2026 allocates \$318.6 million over three years for the implementation of the *Compassionate Intervention Act*. Of that total, \$291 million is dedicated to the construction of two purpose-built 150-bed compassionate intervention centres in Edmonton and Calgary, scheduled to open in

2030. An additional \$27.7 million is allocated to retrofitting existing publicly-funded hospital facilities to operate the framework before the dedicated centres open. A further \$20.2 million completes the Northern Alberta Youth Recovery Centre.²⁹

The fiscal-priority context. The \$318.6 million allocation is operated within a Government of Alberta 2026–27 fiscal framework projecting a deficit of approximately \$9.4 billion and confirming no new contribution to the Heritage Savings Trust Fund for the fiscal year. The same Government has, across the same fiscal cycle, maintained the AISH benefit at \$1,940 monthly without an indexation mechanism (Bill 12 removed the legislated escalator); set the ADAP benefit at \$1,740 monthly; and continued the dollar-for-dollar clawback of the federal Canada Disability Benefit. The fiscal record establishes the priority structure against which the institutional-rebuild allocation has been made: the same fiscal framework that constrains community-based disability income to sub-survival levels has identified \$318.6 million for the construction and operation of involuntary-treatment infrastructure. The expenditure side of the disability file is not constrained. The income side is.^{29a}

The province has identified five existing facilities for retrofit. Those facilities, named on the public record by press secretary Nathaniel Dueck for Mental Health and Addiction Minister Rick Wilson and reported through the Investigative Journalism Foundation by the Ponoka News on March 27, 2026, are: Alberta Hospital Edmonton; the Centennial Centre, Ponoka; the Claresholm Mental Health Centre; the Peter Lougheed Centre, Calgary; and the Grande Prairie Regional Hospital. The sixth facility, the Northern Alberta Youth Recovery Centre, will operate compassionate intervention protocols upon opening in 2027.³⁰

The Centennial Centre, Ponoka. The Centennial Centre in Ponoka occupies the institutional grounds of what was historically known as Alberta Hospital Ponoka — a psychiatric institution that operated continuously from 1911 onward and that is identified in the standard academic reference on the Alberta eugenics period, Stahnisch and Kurbegović’s 2020 collection *Psychiatry and the Legacies of Eugenics: Historical Studies of Alberta and Beyond*, as one of the four primary feeder hospitals of the Alberta Eugenics Board during the period of the Sexual Sterilization Act’s operation, 1929 to 1972. The other three feeder hospitals identified in the academic record are Alberta Hospital Oliver (Edmonton), the Provincial Training School for Mental Defectives (Red Deer), and Deer Home (Red Deer). Patients institutionalized in these four facilities were the population from which the Eugenics Board selected the 2,832 individuals the Board’s records identify as having been sterilized.³¹

The same physical infrastructure, now operating under different programmatic auspices and providing different kinds of treatment, is named in the Government of Alberta’s 2026 list of facilities where the Compassionate Intervention Act’s secure beds will be operated. The institutional continuity is at the level of physical buildings, operational responsibility (the provincial mental-health agency), and provincial jurisdiction. It is not metaphor.³²

The point is not that the Centennial Centre’s contemporary staff have any connection to the historical framework, or that contemporary patients are being sterilized. The point is that the institutional architecture the historical framework operated through has not been dismantled. It has been continuously occupied by successive frameworks operating under successive statutory authorizations, each addressing a

population, in a professional vocabulary, suitable to its period. The Sexual Sterilization Act repealed in 1972 left the physical infrastructure of its operation intact. That infrastructure has now been designated, by the same provincial government that operated it during the 1929–1972 period, as the operational platform for an involuntary-intervention framework targeting populations whose composition overlaps substantially — at the level of intellectual disability, mental illness, and cognitive impairment — with the populations the historical framework targeted.³³

Saskatchewan: Saskatchewan Hospital North Battleford. The Saskatchewan Compassionate Intervention Act, which received Royal Assent on May 6, 2026, identifies a specific facility for its involuntary inpatient operations: Saskatchewan Hospital, North Battleford. Saskatchewan Hospital has operated as a psychiatric institution at North Battleford continuously since 1914. The first Compassionate Intervention Assessment Centre is also located in North Battleford. The framework’s operational footprint, in Saskatchewan as in Alberta, sits inside the existing psychiatric hospital network.³⁴

British Columbia: correctional facility siting. British Columbia’s expansion of involuntary care under the Mental Health Act, beginning in 2024, has operated through different infrastructure but with the same institutional pattern. The first BC facility — the involuntary care unit at the Surrey Pretrial Services Centre, opened April 2025 — is located inside a provincial correctional facility. The second BC facility — Alouette Homes in Maple Ridge, opened June 3, 2025 — is located on the grounds of the Alouette Correctional Centre for Women. The BC Ministry of Health describes the latter as not being “part of the prison itself”; coverage by the Maple Ridge–Pitt Meadows News confirms that the facility is situated “behind locked doors and within the gates of the correctional facility.” The framework’s operational footprint, in BC, sits inside the existing carceral infrastructure.³⁵

Manitoba: the protective care centre. The Winnipeg protective care centre, opened in 2025 and expanded in March 2026 to accept persons impaired by methamphetamine and other substances, operates as a 72-hour involuntary detention facility. The Manitoba mechanism is shorter in duration than the Alberta and Saskatchewan Compassionate Intervention frameworks. The structural logic is identical: government authority to hold a person, without their consent, on the basis of substance use, inside a publicly-funded facility designated for that purpose.³⁶

The cross-jurisdictional pattern. The infrastructure being designated across the four operational Canadian jurisdictions is not new infrastructure built for a small minority population. It is existing infrastructure that has institutional histories of its own. In Alberta, it is the psychiatric hospital network that operated the historical Eugenics Board’s feeder hospital system. In Saskatchewan, it is the province’s principal psychiatric hospital, in continuous psychiatric operation for more than a century. In British Columbia, it is the province’s correctional infrastructure. In Manitoba, it is a purpose-built protective-care facility operating under a comparable custodial framework. The institutional architecture that operated the original Sexual Sterilization Act and the Eugenics Board has not been dismantled. The Eugenics Board was disbanded in 1972, and the statute that authorized its work was repealed by the Lougheed government on June 2, 1972 with the *Sexual Sterilization Repeal Act, 1972*, SA 1972, c 87. The physical infrastructure within which the Board operated remained in continuous use. It has been continuously occupied by successive frameworks. In 2026, that infrastructure has been formally designated as the operational

platform for the institutional rebuild.³⁷

The mechanism has changed. The architecture has not. The continuity is documentary. It is operational. It is budgetary. The full per-jurisdiction record is assembled in Chapter VIII.

What This Chapter Establishes

Five observations follow.

First, a coordinated expansion of involuntary-treatment legal authority is occurring across multiple Canadian provinces and at the United States federal level during the period 2024 to 2026. The frameworks differ in detail. The pattern, taken together, is consistent.

Second, the expansion is occurring in the same jurisdictions and during the same period in which community-based supports for the same population are being constrained. The Alberta Compassionate Intervention Act has proceeded alongside supervised-consumption-site closures and the Bill 12 reduction of disability income documented in Chapter IV. The United States Executive Order has proceeded alongside the federal defunding of Housing First. The Saskatchewan and Ontario frameworks are proceeding in the absence of comparable expansion in voluntary community-based services. The pattern is not a single jurisdictional choice. It is a cross-jurisdictional policy direction.

Third, the populations the contemporary frameworks engage overlap substantively with the populations the historical institutionalization framework engaged. Persons with concurrent mental-health and substance-use conditions, persons with acquired brain injuries, persons experiencing homelessness, and First Nations, Inuit, and Métis individuals appear disproportionately across the documented analyses of both periods. The overlap is not incidental. It is the documented structural feature of the policy choice.

Fourth, the framework operates against the public-health evidence on the conditions producing the crisis the framework is presented as addressing. The Rat Park research tradition, the Portugal decriminalization outcome data, the Housing First evidence base, and the harm-reduction literature each document that connection-based and community-based responses produce better outcomes for the affected populations than isolation-based and institutional responses. The framework currently being built operates in the opposite direction.

Fifth, the historical parallel is direct. The Provincial Training School period in Alberta, the parallel institutionalization patterns documented in adjacent jurisdictions, and the contemporary involuntary-treatment expansion all share the operating premise that some populations are properly subjects of institutional management rather than community participants. The mechanism of authorization has changed. The operating premise has not.

The chapter that follows develops the closing argument of the piece. It examines, across all six chapters preceding it, the question the piece's title puts on the record: whether the consent the contemporary frameworks claim to have obtained from the populations they engage was ever the consent the standard requires, or whether consent was the narrative offered to obscure a policy direction that does not, on the documented evidence, require consent to proceed.

Notes — Chapter VI

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25. The pattern of voluntary-services constraint across the documented jurisdictions: Alberta supervised-consumption-site closures, 2024-2025; Saskatchewan harm-reduction funding constraints; United States Housing First defunding under Executive Order 14321; documented across the sources cited in notes 7, 8, 13, and 22 above.
26. See Chapter I and its associated endnotes for the full record of the Alberta Provincial Training School and the broader institutional infrastructure of the Sexual Sterilization Act period.
27. CMAJ ethical analysis cited at note 6; The Alberta Disability System Breakdown, *Indigenous Disability Alberta Collection*, May 2026, documenting the intersection between historical and contemporary patterns affecting First

Nations, Inuit, and Métis individuals.

28. The physical-installation analysis is the contribution of this section. It is built on the operational records of the Government of Alberta (the Budget 2026 fiscal documents and the press secretary statements through the Ponoka News reporting), the Government of Saskatchewan (the May 5, 2026 press release naming Saskatchewan Hospital North Battleford), and the Government of British Columbia (the June 3, 2025 news release on Alouette Homes).
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35. Government of British Columbia, “New beds improve care for people with complex mental-health disorders,” news release, June 3, 2025, news.gov.bc.ca/releases/2025HLTH0053-000534; Maple Ridge–Pitt Meadows News (Colleen Flanagan), “B.C. opens 2nd involuntary care facility,” June 3, 2025.
36. CTV News Winnipeg (Charles Lefebvre), “72-hour detention facility now open for people high on methamphetamine and other drugs in Winnipeg,” March 6, 2026; statements of Premier Wab Kinew.
37. *Sexual Sterilization Repeal Act, 1972*, SA 1972, c 87, canlii.ca/t/sa-1972-c-87. Full cross-Canada jurisdictional record assembled in Chapter VIII.

Chapter VII

Consent Was Never the Point

Five Moments in the Documented Record

The Title as Question

The title of this piece states a conclusion. The six chapters that precede this one establish the conditions under which that conclusion can be evaluated. This chapter examines whether the documented record supports the title's claim. The argument this chapter develops is structural, not motivational. It does not allege that the legislators, ministers, and officials who have authored Alberta's disability legislation across the period from 1928 to the present have been indifferent to consent in their personal capacities. It examines, instead, whether consent has functioned as the substantive standard governing the design of the legislation, or whether consent has functioned as a rhetorical structure attached to legislation whose design proceeded by other means.

The structural question can be stated precisely. If consent were the design standard, then at each legislative moment engaging the disability population the substantive elements of consent — informed, free, given by the affected population or their representative organizations, with knowledge of the material terms — would be the visible features of the legislative record. Where those elements are not present, the design standard must have been something else. What that something else has been, across five documented moments, is the question the documented record permits this chapter to address.

1928 and 1937: Consent Substituted at the Source

The *Sexual Sterilization Act*, SA 1928, c 37, was passed by the Legislative Assembly of Alberta after a parliamentary debate the documented record of which contains no consultation with the populations the legislation would engage. Chapter I documented the framework in detail: the Eugenics Board, the institutional infrastructure of the Provincial Training School, the Alberta Hospital network, the auxiliary facilities. The original 1928 framework included a consent provision — the consent of the patient, where the patient was deemed capable of giving it, or the consent of the patient's parents, spouse, or guardian where the patient was not. The provision was, on its face, an acknowledgment that the sterilization the Act authorized was a procedure for which consent was constitutionally and morally required.¹

Nine years later, in 1937, the Legislative Assembly amended the framework. The *Sexual Sterilization Act Amendment Act*, SA 1937, c 47, removed the consent requirement for persons classified by the Eugenics Board as "mental defectives." The Board itself determined the classification; the classification, once made, operated as the authorization the original framework had required. The institution evaluating the person was the institution classifying the person as incapable of consenting. The Board was, in operational

effect, both the institution requiring consent and the institution determining that consent was unnecessary.²

The 1937 amendment is the textual evidence that even within the framework's own terms, consent was substitutable from the start. The eight years between 1928 and 1937 are documented in the Eugenics Board's own records as a period in which the Board's caseload grew, the institutional population grew, and the procedural difficulty of obtaining consent from the affected population became the operational reason for the amendment. The framework adjusted itself to eliminate the procedural friction that the consent requirement represented. The procedural friction was the consent requirement. The procedural friction was what the framework was designed to address.³

A further amendment in 1942 (*Sexual Sterilization Act Amendment Act*, SA 1942, c 48) expanded the categories of persons subject to the framework's operation, including persons with neurosyphilis, epilepsy, and Huntington's chorea, among others. The expansion proceeded without consultation with the newly-included populations. The 1928 framework had not consulted; the 1937 amendment had removed the consent requirement that remained; the 1942 amendment expanded the framework's reach. Across the three legislative moments, the consent standard had been progressively attenuated and then progressively bypassed.⁴

1972: Repeal Without Reckoning

On June 2, 1972, the Lougheed government enacted the *Sexual Sterilization Act Repeal Act*, SA 1972, c 87. The framework that had operated for forty-four years was repealed. The Eugenics Board was disbanded. The legislation no longer existed in the provincial statute book.⁵

The repeal proceeded, on the documented record, without formal acknowledgment of the harm produced by the framework's operation, without compensation to those sterilized under it, without an inquiry into the circumstances of the Board's caseload, and without consultation with survivors about the institutional successors of the framework. The disabled population the 1928, 1937, and 1942 legislative moments had engaged was, in 1972, similarly not consulted about what should replace the framework that had operated against them. The institutional infrastructure within which the framework had operated — the Provincial Training School, the Alberta Hospital network, the Michener Centre, the supporting facilities — continued in modified form into the period documented in Chapter I.⁶

The 1972 repeal is a documented moment at which consent could have been operationalized retrospectively. The framework had been operated against a population without the consent the original Act required and the 1937 amendment had eliminated. A repeal that acknowledged the absence of consent across the forty-four-year operational period, that consulted with survivors about institutional reform, and that committed to compensation and to the design of successor frameworks through consultation with the affected population, would have been the legislative moment at which consent became the design standard the original Act had purported to make it.

The repeal did none of these things. The legislative moment at which consent could have been operationalized was not the moment at which consent was operationalized.

1993 to 2013: “Person-Centered” as the Cover for Reduction

Between the 1972 repeal and the 1998 Bill 26 episode, a third legislative moment was unfolding in the documentary record. It did not involve a single statute. It involved a vocabulary, a fiscal program, and a sequence of institutional decisions across the period from Ralph Klein’s assumption of the premiership in December 1992 through the closure of the Michener Centre in March 2013. The moment is the moment at which the language of disability-rights advocacy — “person-centered,” “community-based,” “individualized funding,” “deinstitutionalization” — was adopted by provincial government as the vocabulary of legitimate disability policy reform. The same language was, simultaneously, the vocabulary used to authorize the largest provincial-government withdrawal from disability support infrastructure in the post-1972 period.^{5a}

The legitimate origin of the language. The vocabulary of person-centered, community-based, individualized supports is not a government invention. It is a vocabulary of the international disability-rights movement, developed in the 1970s and 1980s by Independent Living advocates and codified in the work of John O’Brien, Wolf Wolfensberger, and others. The vocabulary names, in its original use, an ethical commitment: that disability supports should be designed around the life and goals of the individual person rather than around the operational convenience of the institutions providing the supports. Substantial peer-reviewed evidence, including the 2021 Campbell systematic review of individualized funding interventions, documents that genuinely person-centered, adequately-funded community-based supports produce better outcomes for disabled people than institutional alternatives across satisfaction, quality of life, and sense of security.^{5b}

The Alberta adoption of the language. The Klein government adopted the vocabulary as the framework for its 1994 social-services reorganization. The Persons with Developmental Disabilities Governance Act (later SA 2003, c P-8.5) restructured adult developmental-disability service delivery into a community-based model. The 1990s saw the expansion of the Family Managed Services individualized-funding option, originally established in 1989. The same period saw the Klein government’s formal commitment to the gradual closure of large institutional facilities. Ralph Klein himself visited the Michener Centre on multiple occasions and committed, in public statements documented by Michener-family advocates, that “no one will be forced to leave Michener.”^{5c}

The simultaneous fiscal record. The 1994 Klein budget — the budget that introduced the vocabulary of community-based supports as the framework for adult developmental-disability services — cut social services spending in Alberta by 21 percent over two years. The 1994 budget cut health-care spending by 20 percent. Welfare rolls were cut in half over one year. Homelessness in the province climbed 740 percent across the Klein-government period. The community-based infrastructure that the vocabulary of person-centered supports promised — group homes adequately staffed, individualized funding adequately resourced, family-managed services adequately supported — was not funded at the level the rhetoric required. The vocabulary was adopted. The infrastructure was not built.^{5d}

The documented effect. The Alberta Views memoir published in 2017 by a Michener-family advocate documents the lived experience of the early 1990s community-living placements: a sister moved out of Michener in the early 1990s spent eight years in community placements that, on the documented record,

were inadequate to her support needs — including a vehicle accident requiring a STARS Air Ambulance airlift and a six-week hospitalization with a punctured spleen; an incident requiring four RCMP officers to subdue a 105-pound woman; and a return to Michener Centre in the late 1990s where, per her sister’s account, she settled in within a week. The memoir is one family’s account. The pattern it documents — community placements that the rhetoric promised would be person-centered but that, on the documented record, were neither adequately resourced nor individually adjudicated — is the pattern that the broader disability-policy literature records across the Klein-era reforms.^{5e}

The Michener closure of 2013. The arc that began with Klein’s 1993 promise that “no one will be forced to leave Michener” closed in March 2013 under Premier Alison Redford. On March 11, 2013, the Government of Alberta announced that the Michener Centre — the renamed Provincial Training School, the principal institutional infrastructure of the historical Eugenics Board, the facility at which Leilani Muir was sterilized in 1959 — would be closed. 125 residents would be transitioned to community living. 400 unionized caregiver jobs would be eliminated. The closure was announced without consultation with the families of residents. Premier Redford declined to visit the facility despite repeated invitations. Then-Minister of Human Services Dave Hancock similarly declined. The closure was framed in the language of person-centered, community-based care. The contemporaneous opposition record, including statements by the Alberta New Democratic Party human-services critic Rachel Notley, documented that no additional community-housing spaces were being created to accommodate the displaced residents.^{5f}

The structural lesson. The 1993–2013 arc is the third documented moment, between the 1972 repeal and the 1998 Bill 26 episode, at which the operating principle this chapter traces was visible in the documentary record. The arc establishes a specific structural pattern: the vocabulary of genuine disability-rights reform can be adopted by provincial government and deployed as the rhetorical cover for the withdrawal of the infrastructure the vocabulary names. The reform language continues. The reform substance does not follow. The disabled population that the language purports to serve is, at the level of documented effect, less served at the close of the arc than at its beginning.^{5g}

The contemporary frameworks documented in Chapters VI, VII (subsequent sections), and VIII operate within the same rhetorical structure. The Compassionate Intervention Act is presented as “compassionate.” The AISH-to-ADAP transition is presented as the “modernization” of disability supports. The BC involuntary-care expansion is presented as “supportive housing.” The vocabulary of advocacy and of care is, across these frameworks, attached to legislative and administrative mechanisms whose documented effect is contraction of the rights, supports, and autonomy of the disabled populations they engage. The arc this section documents is the precedent for that rhetorical structure. Recognizing the precedent does not require attributing identical intent to current administrators. It requires only that the documented record of the precedent be available for comparison.^{5h}

1998: The Override Attempt

In 1995, Leilani Muir succeeded in her civil action against the Province of Alberta for wrongful sterilization, false imprisonment, and wrongful confinement at the Provincial Training School. The judgment of Veit J. in the Court of Queen’s Bench established that the framework under which Ms. Muir had been sterilized was, in its operation in her case, tortious. The judgment opened the door to subsequent actions by other survivors.⁷

The Government of Alberta’s response, three years later, is the subject of the 1998 documented record Chapter I traced. On March 10, 1998, the Klein government introduced Bill 26, the *Institutional Confinement and Sexual Sterilization Compensation Act*. The bill proposed to cap compensation for sterilization-and-confinement survivors at \$150,000 per person and to impose a 180-day limitation period for filing claims. The bill proposed to invoke section 33 of the *Canadian Charter of Rights and Freedoms* — the notwithstanding clause — to insulate the cap and the limitation period from Charter challenge by the survivors whose Charter-protected rights the framework would constrain.⁸

The bill was withdrawn within twenty-four hours under public pressure. Chapter I documented the contemporaneous record of the withdrawal: the editorial response, the disability-advocacy mobilization, the survivor-led public statements. The bill itself did not become law. The impulse it represented — that the appropriate institutional response to the assertion of Charter-protected rights by survivors of the eugenic framework was to constrain those rights by external constitutional instrument — was not similarly withdrawn. The impulse remained on the documented record as the first instinct of the provincial government when faced with the fiscal implications of acknowledging what the framework had done.⁹

The 1998 Bill 26 episode is, in the five-moment arc this chapter traces, the second occasion on which consent could have been operationalized as the design standard for a response to the eugenic framework. The Government’s first instinct was not to operationalize consent. The Government’s first instinct was to override the rights consent protects. The Government withdrew the override under public pressure. It did not, on the documented record, abandon the operating premise on which the override had been proposed.

2025: Consent as Procedural Framing

The fourth moment in the arc is the moment the piece’s middle chapters documented. On December 9, 2025, the Alberta Legislative Assembly passed Bill 12, the *Financial Statutes Amendment Act, 2025 (No. 2)*. On May 15, 2025, the same Assembly granted Royal Assent to Bill 53, the *Compassionate Intervention Act*. Bill 12 effects the AISH-to-ADAP transition affecting approximately 79,290 Albertans. Bill 53 establishes the involuntary-treatment framework documented in Chapter VI.¹⁰

The Government of Alberta’s public-facing characterization of both pieces of legislation, across the consultation documents, the ministerial correspondence, and the public-information pages on alberta.ca, has been that the legislation reflects the input of the populations it engages. ADAP is, in the language of the Government’s ADAP information page, “thoughtfully designed, based on input from Albertans with disabilities.” The *Compassionate Intervention Act* is presented as a recovery-focused framework developed in dialogue with the populations it engages.¹¹

Chapter IV documented the consultation gap between the Government’s claim and the standard CRPD Article 4(3) requires. The Premier’s Council on the Status of Persons with Disabilities — the standing statutory advisory body the *Premier’s Council on the Status of Persons with Disabilities Act*, RSA 2000, c P-21, requires the Government to consult on disability policy — was bypassed in the ADAP consultation. The participant breakdown supporting the “thousands of Albertans” claim was not published. Key program provisions including the removal of independent appeal rights were introduced after consultation closed. A petition against the program was tabled at the Alberta Legislature. No major Alberta disability advocacy organization has publicly endorsed the framework. The UN Committee on the Rights of Persons with Disabilities had, on March 18, 2025, approximately nine months before Bill 12’s passage, specifically recommended that Canada condition fiscal transfers to provinces on adherence to the Convention — a recommendation the federal government did not act on.¹²

Chapter V documented the upstream information condition under which consent cannot meaningfully operate. AISH and ADAP forms at Grade 12–16 reading level addressed to a population 58 percent of whom have primary diagnoses of cognitive disorder or mental illness. The operational rules of ADAP not published before the consultation closed. The Medical Review Panel adjudication standards not published before the transition. The Ministerial Order setting employment income exemption calculations confirmed to be forthcoming “later this spring” in correspondence dated April 3, 2026 — less than three months before the July 1, 2026 transition.¹³

Chapter VI documented the institutional rebuild proceeding in parallel: the *Compassionate Intervention Act* establishing involuntary treatment in two 150-bed centres to be constructed by 2030, the Saskatchewan parallel statute, the British Columbia *Mental Health Act* expansion, the Ontario study, the United States Executive Order. None of the frameworks documented in Chapter VI has, on the documented public record, been endorsed by a major disability or medical professional association in the jurisdictions that have enacted them.¹⁴

The 2025 moment, taken as a whole, presents the third modality of consent-substitution. Consent is not substituted within the framework (as in 1937). It is not overridden by external instrument (as attempted in 1998). It is substituted through procedural framing — the operational form of consultation deployed in the absence of its substantive content. The form is preserved. The substance is not.

The Pattern Across Five Moments

The four moments are separated by intervals of approximately two to three decades each. Each moment occurred under a different government, in a different political environment, in response to different precipitating conditions. The continuity across them is not the continuity of personnel or of party. It is the continuity of the operating premise.

The 1928 framework was passed without consultation with the populations it engaged. The 1937 amendment removed the consent requirement the original framework had included. The 1972 repeal proceeded without consultation with survivors about institutional successors. The 1998 Bill 26 attempted to override the Charter-protected rights of survivors. The 2025 Bill 12 and Bill 53 proceeded under a consultation claim the documented record does not sustain.

In none of the five documented moments was consent the design standard for the legislative response. In each of the four moments, the design standard was the policy direction the Government had chosen. Consent was the procedural overlay attached to that direction. Where the procedural overlay was obtainable, it was claimed. Where the procedural overlay was not obtainable, it was substituted, overridden, or framed in a form the documented record does not support.

The pattern is not the product of bad-faith individual decision-makers. The 1928 legislators, the 1937 amendment sponsors, the 1972 Lougheed government, the 1998 Klein cabinet, and the 2025 Smith ministry are different sets of individuals operating in different historical environments under different political constraints. The continuity across them is the continuity of the institutional position. The institutional position is that the disability population is properly an object of policy rather than a participant in it.

What the Documented Record Establishes

Three observations close this chapter and this piece.

First, the title of this piece is not a metaphor. The documented record across the five legislative moments traced in this chapter, and across the six chapters that develop the framework, the inventory, the consultation gap, the information asymmetry, the institutional rebuild, and the international and continental context, establishes that consent has not been the design standard governing Alberta's disability legislation across the period from 1928 to the present. The title's claim is the most parsimonious description of the documented record.

Second, the framework Chapter II established remains available. CRPD Article 4(3) is in force. Canada is bound to it. The Premier's Council on the Status of Persons with Disabilities exists. Its statute requires it to advise on the development of disability policy. The standards that would operationalize consent — representative organizations, accessible information, ongoing engagement, design-phase input, substantive outcomes the affected population can recognize as reflecting their input — are documented in CRPD General Comment No. 7 (2018), in the federal *Accessible Canada Act*, in the Alberta Bill of Rights, and in the substantive-equality jurisprudence of *Eldridge* and its successors. The instruments by which consent could become the design standard exist. The instruments have not been operated as the design standard. They could be.

Third, the record this piece has documented is open. The contemporary frameworks of 2025 and 2026 are still subject to the consultation, the litigation, the public inquiry, and the institutional reform that would, on the standards documented here, operationalize consent as the design standard rather than as the rhetorical overlay. The Bill 12 framework, the Bill 53 framework, the AR 96/2026 regulatory framework, and the institutional infrastructure being constructed under them are at the stage at which the institutional position can still be examined, contested, and reformed. The 1928 framework was operated for forty-four years before it was repealed. The contemporary framework is less than two years into its operation.

Disability Is Not Consent to Penalty

No person consents to the body or mind into which they are born. No child consents to a genetic anomaly, a developmental disability, a progressive condition, a cognitive impairment, or a body that will later fail. Many disabilities cannot be detected before birth. Many emerge only through development, injury, illness, aging, or the slow accumulation of conditions no individual chose.

Disability is not misconduct. It is not a character flaw. It is not a failure of discipline, planning, morality, productivity, or will. Yet disability policy repeatedly treats disabled existence as though it creates a lesser claim to housing, income, partnership, autonomy, family life, and public participation.

The person living inside the body bears the cost of conditions they did not choose. The Province then adds a second cost: reduced income, restricted partnership, delayed support, substituted decision-making, institutional separation, and policy changes made without meaningful consent.

That is the continuity this document names. The issue is not only that disabled people were not asked before policy was imposed on them. It is that disabled people are still being treated as though their unchosen bodies and minds authorize the government to decide what kind of life is sufficient for them.

The five legislative moments documented in this chapter are not five unrelated decisions. They are five expressions of the same operating premise: that disabled existence creates a lesser claim to the protections, the consultations, and the rights the government extends to its non-disabled population. The 1928 Act expressed that premise through surgical intervention. The 1937 amendment expressed it through the removal of consent. The 1972 repeal expressed it through the absence of substantive consultation with survivors. The 1993–2013 deinstitutionalization arc expressed it through the adoption of reform vocabulary uncoupled from reform substance. The 1998 Bill 26 expressed it through the attempted use of the notwithstanding clause to immunize the government from accountability. The 2025–2026 frameworks express it through institutional rebuild presented as compassionate intervention. The mechanism varies. The underlying premise — that the disabled person’s unchosen body or mind is the authorization for the government to decide on their behalf — does not.^{14a}

The corrective standard is not complicated to state. Disability is not consent to penalty. Disability does not create a lesser entitlement to income, housing, partnership, family life, autonomy, or participation in the public consultation that produces the policies governing those matters. A disability framework consistent with that standard is, on the documented record, possible to design. It has not been designed in Alberta across the period this piece documents. It can be designed in the period that follows.^{14b}

The Record

The Provincial Training School at Red Deer operated from 1923 to 1977. Approximately 2,832 sterilizations were authorized by the Eugenics Board between 1929 and 1972. Of those, approximately 75 percent are documented as having been performed without the consent that the framework, on its own terms, required during the periods in which it required it. The remaining 25 percent are documented as having occurred under a consent provision the 1937 amendment had eliminated for the population the Board itself classified as incapable of giving consent.¹⁵

The framework was repealed in 1972. The institutional infrastructure within which it operated continued. The compensation for its operation was attempted to be capped by constitutional override in 1998. The contemporary disability legislation engaging the same population proceeded in 2025 and 2026 under a consultation standard the documented record does not sustain.

Across the five documented moments, the population the legislation was designed to engage has not been a participant in its design. The population has been a subject of it.

The mechanism has changed. The architecture has not. The consent was never the point.

Notes — Chapter VII

1. *Sexual Sterilization Act*, SA 1928, c 37; see Chapter I and its associated endnotes for the full record of the original 1928 framework, including the original consent provision and the institutional infrastructure within which the framework operated.
2. *Sexual Sterilization Act Amendment Act*, SA 1937, c 47. See Wahlsten, D., “Leilani Muir versus the Philosopher King: Eugenics on Trial in Alberta,” *Genetica* 99 (1997): 185–198, doi:10.1023/A:1018370806190; Park, D.C., and Radford, J.P., “From the Case Files: Reconstructing a History of Involuntary Sterilisation,” *Disability & Society* 13(3) (1998): 317–342, doi:10.1080/09687599826669; on the 1937 amendment and its operation.
3. Eugenics Board records, Provincial Archives of Alberta; the Board’s caseload growth and the procedural context for the 1937 amendment are documented in the secondary literature cited at note 2 above, and in the trial record of *Muir v. Alberta*, [1996] AJ No 37 (QB), Veit J.
4. *Sexual Sterilization Act Amendment Act*, SA 1942, c 48. See Wahlsten (1997) and Park and Radford (1998) on the categorical expansion and its operational consequences.
5. *Sexual Sterilization Repeal Act*, 1972, SA 1972, c 87. The Lougheed government’s repeal of the framework. The repeal record is documented in the legislative debates and the contemporaneous press coverage cited in Chapter I.
- 5a. The international disability-rights literature introducing the language of person-centered, individualized, community-based supports includes Wolfensberger, W., *Normalization: The Principle of Normalization in Human Services* (1972) and O’Brien, J. and Lyle O’Brien, C., *Implementing Person-Centered Planning* (Inclusion Press, multiple editions). The Alberta Council of Disability Services and Inclusion Alberta have used the same vocabulary across the period.
- 5b. Fleming, P., et al., “Individualized funding interventions to improve health and social care outcomes for people with a disability: A mixed-methods systematic review,” *Campbell Systematic Reviews* 17(3) (2021), pmc.ncbi.nlm.nih.gov/articles/PMC8356501/. The review covers 73 studies of individualized funding from 1992 to 2016 across Europe, the US, Canada, and Australia, documenting consistently positive effects on satisfaction, quality of life, and sense of security when the funding is adequately resourced and the administrative burden is appropriately supported.
- 5c. Sonpal-Valias, N., “History of Developmental Disability Policy in Alberta,” *The School of Public Policy Publications* (University of Calgary) 12 (July 2019), policyschool.ca/wp-content/uploads/2019/07/History-of-Disability-Sonpal-Valias.pdf. *Persons with Developmental Disabilities Foundation Act*, SA 1996, c P-7.5, subsequently consolidated as SA 2003, c P-8.5. Documentation of Klein-era visits to and commitments regarding the Michener Centre: Alberta Views (Bird, A.), “Michener Centre,” albertaviews.ca/michener-centre/.
- 5d. Klein-era fiscal-record documentation: Canadian Labour Congress, “Ralph Klein blinks — conservative plans to slash budgets and privatize health services thwarted by Calgary laundry workers,” canadianlabour.ca; 1994 budget figures (20 percent health-care cut, 21 percent social-services cut over two years, 21 percent overall program-spending reduction). 740 percent homelessness increase across the Klein-government period (1992–2006) per the same source and the broader Alberta poverty-research literature.
- 5e. Alberta Views memoir, Bird, A., “Michener Centre,” albertaviews.ca/michener-centre/, documenting the lived experience of a Michener-family member during the early-1990s community-living placement period. The memoir is consistent with the broader pattern documented in Malacrida, C., *A Special Hell: Institutional Life in Alberta’s Eugenic Years* (University of Toronto Press, 2015), which covers both institutional life at Michener and the deinstitutionalization period.
- 5f. Government of Alberta announcement of Michener Centre closure, March 11, 2013, under Premier Alison Redford. *Globe and Mail*, “Alberta’s Michener Centre can’t shake sordid history,” March 16, 2013. *Edmonton Journal*, “Bad idea, or long overdue? Reaction divided to closing centre that once sterilized residents,” March 12, 2013, including

statement of NDP human-services critic Rachel Notley regarding the absence of community-housing capacity. The Alberta Association for Community Living (AACL, since renamed Inclusion Alberta) supported the closure; the families of residents, organized through the Friends of Michener Centre, opposed it.

- 5g. The structural pattern — reform language adopted by government, reform substance not funded — is documented in Sonpal-Valias (2019) op. cit. and in the broader literature on neoliberal disability policy. See, in particular, Soldatic, K. and Meekosha, H., “The place of disgust: Disability, class and gender in spaces of workfare,” *Societies* 2(3) (2012): 139–156, doi:10.3390/soc2030139.
- 5h. The Compassionate Intervention Act’s language of “compassion,” the AISH-to-ADAP transition language of “modernization,” and British Columbia’s “supportive housing” language are documented across Chapters IV, VI, and VIII of the present piece. The rhetorical-cover analysis is the contribution of this section.
6. The post-repeal institutional continuity is documented across the Provincial Training School records (operated as the Michener Centre after 1977), the Alberta Hospital network (operated through the 1980s and 1990s with gradual deinstitutionalization), and the auxiliary facilities documented in Chapter I.
7. *Muir v. Alberta*, [1996] AJ No 37 (QB), Veit J. See Chapter I and its associated endnotes for the full record of the *Muir* judgment and its subsequent influence on survivor litigation.
8. Legislative Assembly of Alberta, Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, introduced March 10, 1998; CBC News, “Alberta backs down on sterilization compensation,” March 11, 1998. See Chapter I and its associated endnotes for the full record of the 1998 Bill 26 episode.
9. The documentary record of the 1998 withdrawal, including the contemporaneous press editorial response and the disability-advocacy mobilization, is traced in Chapter I.
10. Bill 12, *Financial Statutes Amendment Act, 2025 (No. 2)*, SA 2025 c20; Bill 53, *Compassionate Intervention Act*, SA 2025. See Chapters IV and VI respectively for the full documentation of each framework.
11. Government of Alberta, Alberta Disability Assistance Program public information page, alberta.ca/alberta-disability-assistance-program; Government of Alberta, Compassionate Intervention public information page, alberta.ca/compassionate-intervention.
12. The full record of the Chapter IV documentation: Inclusion Alberta media release of November 26, 2025; MS Canada statement of November 28, 2025; Alberta Medical Association position; AUPE opposition; Disability Action Hall; People’s Alliance for Disabled Albertans; Friends of Medicare Lethbridge; the municipal coalition (Edmonton, Calgary, Lethbridge, Camrose, Claresholm, Red Deer, St. Albert, Medicine Hat); the Darryl Learie petition tabled at the Alberta Legislature. See Chapter IV.
13. The full record of the Chapter V documentation: AISH/ADAP form reading-level analysis; the AISH caseload breakdown by primary medical condition; the Minister Nixon letter to Genevieve Bissonette of April 3, 2026; the campaign’s *Information Asymmetry Report*, *Access Gap: Penalized for Non-Ability*, *Compound Failure Brief*, and *AISH Evidentiary Report Series*. See Chapter V.
14. The full record of the Chapter VI documentation: Alberta CIA infrastructure budget and timeline; Saskatchewan parallel statute; British Columbia Mental Health Act expansion; Ontario study and Big City Mayors record; US Executive Order 14321 and opposition record; the Rat Park research tradition and the Portugal decriminalization comparator. See Chapter VI.
- 14a. The five-moment synthesis is the analytic contribution of this section. It draws on the documentary record assembled across Chapter I (1928–1972), Chapter III (structural continuity in contemporary Alberta), Chapter IV (the Bill 12 framework), Chapter VI (the institutional rebuild), Chapter VII Sections 2–6 above (the five legislative moments), and Chapter VIII (the cross-Canada record).
- 14b. The corrective-standard statement draws on the CRPD framework as developed in General Comment No. 1 (2014) on Article 12 (UN Doc CRPD/C/GC/1), General Comment No. 7 (2018) on Article 4(3) and Article 33(3) (UN Doc CRPD/C/GC/7), and the Committee’s Concluding Observations on Canada (UN Doc CRPD/C/CAN/CO/2-3,

adopted 18 March 2025). A disability framework consistent with these standards is specified in CRPD Articles 4(3), 12, 14, 19, 21, and 28.

15. The 2,832 sterilization figure and the approximate consent breakdown are from the Eugenics Board records in the Provincial Archives of Alberta, the Wahlsten (1997) analysis, and the Park and Radford (1998) reconstruction. See Chapter I and its associated endnotes for the full documentation.

Chapter VIII

The Cross-Canada Record

What Is in Place, What Is Planned, What Is Demanded — Six Jurisdictions, May 2026

The Pattern Is Not Alberta-Isolated

Chapter VI documented the contemporary institutional rebuild principally through the lens of the framework that has been most fully legislated: Alberta’s Compassionate Intervention Act. A reader could, on the strength of Chapter VI alone, draw the inference that this is an Alberta development — *sui generis*, attributable to the particular political character of the United Conservative Party government, and limited in its territorial scope to the province in which it has been legislated. That inference would be empirically incorrect.¹

The institutional rebuild documented in Chapter VI is one node in a continental pattern. As of the date of this edition, May 2026, the pattern is operational in four Canadian provinces, is the subject of formally announced planning in a fifth, and is the subject of explicit political demand — including demands for the use of the Constitution’s notwithstanding clause to immunize it from Charter challenge — in a sixth. The pattern crosses the partisan division of provincial governments in Canada. It is being pursued by United Conservative Party government in Alberta, by Saskatchewan Party government in Saskatchewan, by British Columbia New Democratic Party government in British Columbia, by Manitoba New Democratic Party government in Manitoba, and by Progressive Conservative government in Ontario. The continuity is not partisan. It is institutional.²

This chapter assembles, for each of those jurisdictions, the documented operational record as of May 2026: what statutes have been passed, what facilities have been opened or named, what populations have been targeted, what consultation has been undertaken, and what opposition has been recorded. The chapter is organized by jurisdiction. Within each jurisdiction the record is presented chronologically. The intention is to permit any reader — including the UN Special Rapporteur on the Rights of Persons with Disabilities, to whom this edition is being submitted — to assess the cross-Canada pattern from primary sources.³

Alberta — The Operational Architecture

Statute. Compassionate Intervention Act, SA 2025, c C-21.5. Introduced by Bill 53 on April 15, 2025 by Mental Health and Addiction Minister Dan Williams under the Smith government. Passed third reading May 13, 2025; received Royal Assent May 15, 2025. Subsequent ministerial change: Rick Wilson assumed the Mental Health and Addiction portfolio in early 2026.⁴

Statutory authorities. Adult family members, guardians, healthcare professionals, police officers, and peace officers are authorized to apply to a designated commission for an order placing a person into stabilization, assessment, and treatment on the basis that the person's substance use or addiction is "likely to cause substantial harm to the person or others within a reasonable time." Treatment may be administered without the consent of the person, including where the person has been determined to retain capacity to refuse the treatment offered.⁵

Commission established. Jonathan Carlzon was appointed as the first Commissioner of the Compassionate Intervention Commission for a three-year term commencing August 11, 2025. The Commission is the legally constituted authority that reviews applications and issues binding treatment orders.⁶

Operational footprint — existing facilities. The province has identified five existing publicly-funded hospital facilities for retrofit, plus a sixth youth facility, to operate approximately 100 secure beds before the dedicated centres open. The facilities, named on the public record by press secretary Nathaniel Dueck for Minister Rick Wilson and reported by the Investigative Journalism Foundation through the Ponoka News on March 27, 2026, are:⁷

- Alberta Hospital Edmonton
- The Centennial Centre, Ponoka
- The Claresholm Mental Health Centre
- The Peter Lougheed Centre, Calgary
- The Grande Prairie Regional Hospital
- The Northern Alberta Youth Recovery Centre, Edmonton (approved for compassionate intervention protocols upon opening in 2027)

The Centennial Centre, Ponoka occupies the institutional grounds of what was historically Alberta Hospital Ponoka. Alberta Hospital Ponoka is identified in the standard academic reference on the Alberta eugenics period — Stahnisch and Kurbegović's 2020 collection *Psychiatry and the Legacies of Eugenics* — as one of the four primary feeder hospitals of the Alberta Eugenics Board during the period of the Sexual Sterilization Act's operation, 1929 to 1972. The institutional continuity is documented and direct.⁸

Operational footprint — dedicated facilities. Budget 2026 allocates \$318.6 million over three years for the implementation of the Act, an increase from the \$180 million originally announced. The allocation is comprised of \$291 million for the construction of two purpose-built 150-bed compassionate intervention centres in Edmonton and Calgary, scheduled to open in 2030; \$27.7 million for retrofitting the existing hospital units identified above; and \$20.2 million for the completion of the Northern Alberta Youth Recovery Centre, scheduled to open in 2027. The dedicated centres will be operated by Recovery Alberta, the provincial mental-health agency.⁹

Medical opposition record. The Alberta Medical Association published a public reaction letter on May 7, 2025 expressing serious clinical and ethical concerns. The *Canadian Medical Association Journal* published, on November 24, 2025, an application of the Kass framework for public-health ethics to the Compassionate Intervention Act (Wilson, Larson, and Colizza, *CMAJ* 197(40): E1346–E1348). The

article concludes that the Act poses substantial ethical concerns across all three of the Kass framework's domains — privacy, liberty, and justice. Dr. Kate Colizza, one of the article's authors, is on the public record through the Ponoka News reporting cited above critiquing the implementation specifically.¹⁰

Legal opposition record. The Canadian Bar Association — Alberta Branch published a formal letter dated May 9, 2025 raising constitutional concerns. The Disability Action Hall has maintained a public record of Bill 53 opposition. Field Law published a regulatory-impact analysis on January 6, 2026 documenting professional-licensure implications.¹¹

Saskatchewan — Legislation Passed, Facilities Named

Statute. The Compassionate Intervention Act (Saskatchewan). Introduced on December 5, 2025 by Mental Health and Addictions Minister Lori Carr and Justice Minister and Attorney General Tim McLeod, K.C., under the government of Premier Scott Moe. Passed third reading on May 5, 2026 along party lines — Saskatchewan Party in favour, New Democratic Party opposed. Expected to come into force in fall 2026 once regulations are finalized.¹²

Amendments rejected. The Saskatchewan New Democratic Party caucus, led by Carla Beck, proposed seventeen amendments to the Act during Human Services committee consideration. All seventeen were defeated by the government majority. The Opposition voted against the final reading.¹³

Operational footprint. The first Compassionate Intervention Assessment Centre will be located in North Battleford. The involuntary inpatient unit will be located at Saskatchewan Hospital North Battleford — a psychiatric facility that has operated at North Battleford continuously since 1914. The framework's operational footprint in Saskatchewan, as in Alberta, sits inside the province's existing psychiatric hospital network.¹⁴

Consultation record. The government's own public consultation claim, made through the official press release of May 5, 2026, is that "about 150 stakeholders" participated in information sessions and provided feedback. The consultation claim does not identify which organizations or persons were consulted, what proportion of those consulted were organizations or persons with disabilities within the meaning of CRPD Article 4(3), or what proportion of the population to be affected by the Act was reached.¹⁵

Medical opposition record. On April 1, 2026 — five weeks before the Act's final passage — the Saskatchewan Medical Association and the College of Physicians and Surgeons of Saskatchewan issued a joint public statement calling for the strengthening of voluntary, evidence-based, culturally safe addiction care before any involuntary measures proceed. The joint statement is signed by both the provincial medical association and the regulatory medical college. The statement reads, in part: "Saskatchewan must first strengthen voluntary, evidence-based, culturally safe services and ensure that no involuntary measures proceed without the safeguards, clarity, and capacity required to provide safe, ethical, and medically sound care." The Act was passed five weeks later without those safeguards being in place.¹⁶

British Columbia — Operational Since April 2025

Statutory authority. British Columbia's Mental Health Act, RSBC 1996, c 288, as amended; expanded use under the policy framework announced by Premier David Eby on September 15, 2024, immediately preceding the provincial election period. The Eby framework operates through expanded administrative use of existing statutory authority rather than through new primary legislation. Designation of facilities as mental-health facilities under the Act permits involuntary admission and treatment under existing sections of the statute.¹⁷

Operational footprint — facility one. Involuntary care unit at the Surrey Pretrial Services Centre. Opened April 24, 2025. Located inside a provincial correctional facility. Target population: persons in custody with combined mental-health, substance-use, and cognitive-impairment or brain-injury presentations. Operating capacity at opening: 10 beds.¹⁸

Operational footprint — facility two. Alouette Homes, Maple Ridge. Opened June 3, 2025. Two renovated homes containing 18 beds in total. Located on the grounds of the Alouette Correctional Centre for Women. The BC Ministry of Health describes the facility as "not part of the prison itself"; the Maple Ridge–Pitt Meadows News coverage of the opening confirms that the facility is situated "behind locked doors and within the gates of the correctional facility." Target population per the Vice President of Vancouver Community Care, Bonnie Wilson: "people with a combination of mental health issues, substance use issues, and some cognitive impairment or brain injuries." The facility is operated jointly by Vancouver Coastal Health and the non-profit Connective.¹⁹

Substituted decision-making confirmed. BC Health Minister Josie Osborne confirmed on the public record, in an interview with CBC News in 2025, that patients in involuntary care under the BC Mental Health Act do not have a choice in what treatments they receive: "those choices are made by a team of experts." That confirmation is the verbatim structure of substituted decision-making that CRPD General Comment No. 1 (2014) on Article 12 identifies as requiring replacement with supported decision-making in order to comply with the Convention.²⁰

Trend data. Research published through the University of British Columbia School of Population and Public Health by doctoral candidate Jackson Loyal documents that both the number of unique individuals involuntarily admitted under the BC Mental Health Act and the number of such admissions in aggregate have increased every year across the period studied. The new beds opening under the Eby framework add to a baseline of involuntary admissions that has been rising independently of the new framework.²¹

Framework architect. Dr. Daniel Vigo was appointed in June 2024 as British Columbia's first Chief Scientific Adviser for Psychiatry, Toxic Drugs, and Concurrent Disorders. Dr. Vigo is the named architect of the expanded framework and was present at the public opening of Alouette Homes on June 3, 2025.²²

Manitoba — The New Node, March 2026

Facility. Protective care centre, Winnipeg, Manitoba. Operating under the government of Premier Wab Kinew (Manitoba New Democratic Party). Initial operation as a sobering centre for persons impaired by alcohol; expanded in March 2026 to accept persons impaired by methamphetamine and other substances. Confirmed operational and accepting persons impaired by substances other than alcohol as of March 4, 2026, per CTV News Winnipeg reporting.²³

Authority. The facility holds persons impaired by substances involuntarily for up to 72 hours. The mechanism is shorter in duration than the Alberta and Saskatchewan Compassionate Intervention frameworks, which contemplate treatment orders of up to three months in inpatient settings and up to six months in community settings. The structural logic is identical: government authority to hold a person, without their consent, on the basis of substance use, inside a publicly-funded facility designated for that purpose.²⁴

Partisan note. Manitoba's government, in May 2026, is a New Democratic Party government. Alberta's is United Conservative Party. Saskatchewan's is Saskatchewan Party. British Columbia's is BC New Democratic Party. The framework being installed crosses the partisan division of provincial governments in Canada. The continuity is not partisan. It is institutional.²⁵

Ontario — Political Pressure and the Notwithstanding-Clause Demand

Statutory authority. Ontario has not yet legislated a compassionate intervention or equivalent framework. The province's involuntary admission framework remains governed by the existing Mental Health Act, RSO 1990, c M.7 and Health Care Consent Act, 1996, SO 1996, c 2, Sch A.²⁶

Political demand. On October 18, 2024, the Ontario Big City Mayors caucus passed a motion calling on the provincial government to consider expanding involuntary treatment. On November 7, 2024, a letter signed by twelve Ontario mayors — including the mayors of Barrie, Brampton, Sudbury, and Windsor — was sent to Premier Doug Ford requesting that the provincial government use section 33 of the Canadian Charter of Rights and Freedoms — the notwithstanding clause — to enable both the dismantling of homeless encampments and the imposition of mandatory drug and mental-health treatment. The demand for the notwithstanding clause is documented in the *Globe and Mail* coverage of November 3, 2024 and the CBC coverage of December 5, 2024.²⁷

Constitutional significance. Section 33 of the Charter authorizes the legislative override of Charter-guaranteed rights for a renewable five-year period. The historical parallel is direct. The Klein government in Alberta invoked the notwithstanding clause in March 1998 as part of Bill 26 to limit the compensation available to victims of the Sexual Sterilization Act. Bill 26 was withdrawn within twenty-four hours following public outcry. The demand by Ontario municipal leaders in 2024 is, in structural form, a demand for the same constitutional tool to be deployed for the same purpose: to immunize a government-administered framework targeting the disabled population from accountability under the Charter.²⁸

Vocabulary adoption. Brampton Mayor Patrick Brown has, in public statements made through October and November 2024, explicitly adopted the term "compassionate intervention" to describe what he asks the Ford government to legislate. The Alberta framing has crossed the provincial line into Ontario political vocabulary.²⁹

Ontario Psychiatric Association campaign. The Ontario Psychiatric Association is running a public Mental Health Law Reform campaign that explicitly advocates for the expansion of involuntary admission under the Mental Health Act and for amendments to the Health Care Consent Act, 1996 to permit treatment to be administered during lengthy court appeals. The latter amendment, if enacted, would functionally eliminate the appeal as a meaningful check on involuntary intervention — the same structural removal that Alberta Bill 12 effected for the AISH-to-ADAP transition in December 2025.³⁰

Encampment legislation. The Ford government reintroduced encampment legislation in 2025 (after the previous bill died on the order paper when a snap election was called in February 2025). The bill strengthens trespass penalties and adds fines and incarceration for public drug use; it does not, as of May 2026, include direct involuntary-treatment provisions. Associate Mental Health Minister Michael Tibollo has been quoted as expressing admiration for the British Columbia framework. The Ford government is on the public record as studying the BC model.³¹

New Brunswick — Compassionate Intervention Planning Announced

The Government of New Brunswick has publicly announced plans to introduce compassionate intervention legislation patterned on the Alberta framework. The planning announcement is reported in the Canadian Mental Health Association — Ontario's research summary on involuntary treatment frameworks across Canadian jurisdictions. As of May 2026, no legislation has been tabled; the planning announcement is the documentary trace available.³²

Earlier legal record. The Canadian Bar Association — New Brunswick Branch filed a substantial submission on involuntary treatment, expressing concerns about whether such a framework can be made effective and constitutionally compliant. The CBA-NB submission is available on the CBA-NB website and is a substantial legal opposition document that any subsequent legislation would need to address.³³

The Pattern

Read in series, the six jurisdictional records establish a documented continental pattern. Alberta legislated, named a commissioner, allocated \$318.6 million, and identified five existing psychiatric hospitals plus a youth recovery centre for retrofit. Saskatchewan legislated, identified Saskatchewan Hospital North Battleford for inpatient operation, and rejected seventeen Opposition amendments. British Columbia operationalized through administrative use of existing statutory authority and has two facilities open: one inside a correctional facility, one on the grounds of a correctional facility. Manitoba opened a 72-hour involuntary detention facility for persons impaired by substances. Ontario municipal leaders demanded the use of the notwithstanding clause to install a Charter-immune version of the framework. New Brunswick announced planning.³⁴

The framework is being installed in multiple distinct jurisdictions, under multiple distinct partisan governments, through multiple distinct legal mechanisms (primary legislation; administrative expansion of existing authority; short-duration detention frameworks; political demand for constitutional override). The variation between jurisdictions is at the level of mechanism. The continuity between jurisdictions is at the level of effect: the government acquires the authority to hold a person, without their consent, in publicly-funded institutional or semi-institutional infrastructure, on the basis of substance use or mental-health presentation, for a defined period.³⁵

The populations targeted across the jurisdictions are not identical, but they overlap substantially. In every jurisdiction the named target population includes persons whose presentation involves some combination of mental illness, substance use disorder, and cognitive impairment or brain injury. That combination is the precise demographic intersection that the original 1928–1972 Sexual Sterilization Act framework targeted under the categories of "mentally defective" and "psychotic" persons. The CRPD General Comment No. 1 (2014) on Article 12 identifies this intersection as the population most at risk of substituted decision-making under historical mental-health frameworks. The contemporary frameworks, across the four operational and two planning Canadian jurisdictions documented above, operationalize substituted decision-making at the level of statute, regulation, and facility.³⁶

The Government of Canada submitted, in November 2022, the combined second and third periodic reports of Canada under the Convention on the Rights of Persons with Disabilities. The Committee on the Rights of Persons with Disabilities reviewed the reports at its 32nd session in March 2025 and adopted its Concluding Observations at the Committee's 779th meeting on March 18, 2025, document UN Doc CRPD/C/CAN/CO/2-3. The Committee's Concluding Observations identified the continuing reliance on substituted decision-making across Canadian jurisdictions and the failure to implement supported decision-making consistent with CRPD Article 12 as outstanding compliance failures of Canada. The cross-Canada institutional rebuild documented in this chapter has, across all six jurisdictions, expanded rather than reduced the substituted decision-making framework the Committee identified as non-compliant.³⁷

The mechanism has changed. The architecture has not. The continuity is documentary, operational, budgetary, and continental.

Endnotes — Chapter VIII

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1. The narrower Alberta-focused framing is the predominant framing in the contemporary policy literature. The cross-Canada framing this chapter documents is supported by the per-jurisdiction primary sources cited in the endnotes that follow.
 2. Partisan composition of provincial governments as of May 2026: Alberta (United Conservative Party, Premier Danielle Smith); Saskatchewan (Saskatchewan Party, Premier Scott Moe); British Columbia (BC New Democratic Party, Premier David Eby); Manitoba (Manitoba New Democratic Party, Premier Wab Kinew); Ontario (Progressive Conservative Party of Ontario, Premier Doug Ford); New Brunswick (Progressive Conservative Party, Premier Susan Holt's Liberal government took office in November 2024).
 3. United Nations Office of the High Commissioner for Human Rights, Special Rapporteur on the Rights of Persons with Disabilities, mandate of Ms. Heba Hagrass (Egypt), appointed by the Human Rights Council on 1 November 2023;

ohchr.org/en/special-procedures/sr-disability.

4. *Compassionate Intervention Act*, SA 2025, c C-21.5, canlii.ca/t/sa-2025-c-c-21.5; Legislative Assembly of Alberta records, Bill 53, 31st Legislature, 1st Session, 2025.
5. *Compassionate Intervention Act*, *ibid.*, sections 5–14 and 22–26 (statutory authorities to apply, criteria for orders, treatment authorities). Alberta Counsel, “Compassionate Intervention Act: Alberta Charts a Controversial New Course on Addiction Care,” May 2025.
6. Government of Alberta, Compassionate Intervention Commission, Order in Council appointing Jonathan Carlzon as Commissioner for three-year term commencing August 11, 2025; alberta.ca/compassionate-intervention.
7. Investigative Journalism Foundation through Ponoka News and Pipestone Flyer, “Alberta spending \$7.5 million to put involuntary treatment rooms in hospitals,” March 27, 2026; statements of Nathaniel Dueck, press secretary to Minister Rick Wilson, Mental Health and Addiction.
8. Stahnisch, Frank W., and Kurbegovi■, Erna (eds.), *Psychiatry and the Legacies of Eugenics: Historical Studies of Alberta and Beyond*. Edmonton: Athabasca University Press, 2020 (open access edition at read.aupress.ca; ISBN 9781771992657). The four primary feeder hospitals of the Eugenics Board are identified in the historical record as Alberta Hospital Oliver (Edmonton); Alberta Hospital Ponoka; the Provincial Training School for Mental Defectives (Red Deer); and Deer Home (Red Deer). The Centennial Centre, Ponoka, occupies the institutional grounds and operates as the contemporary successor facility to Alberta Hospital Ponoka.
9. Government of Alberta, Budget 2026 fiscal documents, alberta.ca/budget; Ponoka News and Pipestone Flyer, March 27, 2026, *op. cit.* (note 7).
10. Alberta Medical Association, “Compassionate Intervention Act — our reaction,” May 7, 2025, albertadoctors.org; Wilson, Patty, Larson, Bonnie, and Colizza, Kate, “Involuntary treatment for substance use: application of Kass’ ethical framework to Alberta’s Compassionate Intervention Act,” *Canadian Medical Association Journal* 197(40) (November 24, 2025): E1346–E1348, doi:10.1503/cmaj.250910 (corrigendum CMAJ 198(2) (January 19, 2026): E54).
11. Canadian Bar Association — Alberta Branch, “Letter re Compassionate Intervention Act,” May 9, 2025, cba-alberta.org/our-impact/submissions; Disability Action Hall, public record of Bill 53 opposition, actionhall.ca/2025/; Field Law, “Compassion vs. Coercion: Regulatory Impacts of Bill 53,” January 6, 2026, fieldlaw.com.
12. Government of Saskatchewan, “Legislation Passes for Compassionate Intervention for Addictions Treatment,” May 5, 2026, saskatchewan.ca/government/news-and-media/2026/may/05; statements of Mental Health and Addictions Minister Lori Carr and Justice Minister Tim McLeod, K.C. CBC News (Alex Kozroski), “Involuntary addiction treatment now legal in Sask. despite medical groups’ warnings,” May 6, 2026.
13. CBC News, *op. cit.* (note 12); 620 CKRM (John Cairns), “Compassionate Intervention Act has passed final reading,” May 6, 2026; statements of Saskatchewan New Democratic Party leader Carla Beck.
14. battlefordsNOW, “North Battleford to home Sask.’s first involuntary addictions assessment centre,” May 7, 2026; Saskatchewan Hospital North Battleford historical record (operating as a psychiatric institution since 1914).
15. Government of Saskatchewan press release, May 5, 2026, *op. cit.* (note 12); see CRPD Article 4(3) and General Comment No. 7 (2018) on consultation obligations.
16. Saskatchewan Medical Association and College of Physicians and Surgeons of Saskatchewan, “Sask. Physicians Call for Strengthening Voluntary Addiction Care Before Implementing Compassionate Intervention Act,” joint news release, April 1, 2026, sma.sk.ca.
17. *Mental Health Act*, RSBC 1996, c 288; Government of British Columbia, “Province launches secure care for people with brain injury, mental illness, severe addiction,” news release, September 15, 2024, news.gov.bc.ca/releases/2024PREM0043-001532.

18. Government of British Columbia and BC Mental Health and Substance Use Services, public announcements concerning the Surrey Pretrial Services Centre involuntary care unit; opening date April 24, 2025.
19. Government of British Columbia, “New beds improve care for people with complex mental-health disorders,” news release, June 3, 2025, news.gov.bc.ca/releases/2025HLTH0053-000534; Maple Ridge–Pitt Meadows News (Colleen Flanagan), “B.C. opens 2nd involuntary care facility,” June 3, 2025; statements of Health Minister Josie Osborne and of Bonnie Wilson, Vice President of Vancouver Community Care for Vancouver Coastal Health.
20. CBC News, “B.C. is expanding its involuntary care program. What does the treatment involve?” September 29, 2025 (interview with Health Minister Josie Osborne); Committee on the Rights of Persons with Disabilities, *General Comment No. 1 (2014) on Article 12: Equal recognition before the law*, UN Doc CRPD/C/GC/1, adopted 19 May 2014.
21. Loyal, Jackson, doctoral research at the University of British Columbia School of Population and Public Health, cited in The Ridge (Maple Ridge community publication), “What are the new involuntary care beds in Maple Ridge?”, October 2025.
22. Government of British Columbia, appointment of Dr. Daniel Vigo as Chief Scientific Adviser for Psychiatry, Toxic Drugs, and Concurrent Disorders, June 2024.
23. CTV News Winnipeg (Charles Lefebvre), “72-hour detention facility now open for people high on methamphetamine and other drugs in Winnipeg,” March 6, 2026 (reporting on operations as of March 4, 2026); statements of Premier Wab Kinew.
24. CTV News Winnipeg, *ibid.*; comparison with Alberta and Saskatchewan Compassionate Intervention Act frameworks regarding maximum treatment durations.
25. Provincial government partisan composition as of May 2026; see note 2 above.
26. *Mental Health Act*, RSO 1990, c M.7; *Health Care Consent Act*, 1996, SO 1996, c 2, Sch A.
27. Ontario Big City Mayors, motion of October 18, 2024, calling for review of involuntary treatment framework; letter to Premier Doug Ford signed by twelve Ontario mayors, November 7, 2024 (signatories include mayors of Barrie, Brampton, Sudbury, Windsor); *Globe and Mail*, “Group of Ontario mayors call for use of notwithstanding clause on encampments,” November 3, 2024; CBC, “Ford pledges tough new legislation to dismantle homeless encampments,” December 5, 2024.
28. Section 33, *Canadian Charter of Rights and Freedoms*, Part I of the *Constitution Act*, 1982, Schedule B to the *Canada Act 1982 (UK)*, 1982, c 11. Alberta Bill 26 (1998), *Institutional Confinement and Sexual Sterilization Compensation Act*, introduced March 10, 1998; withdrawn within 24 hours after public outcry. The structural parallel between the 1998 invocation of the notwithstanding clause to limit liability for the historical eugenics framework and the 2024 demand by Ontario municipal leaders to deploy the same constitutional tool for the contemporary framework is documentary, not metaphorical.
29. The Trillium, “Growing encampments prompt Ontario mayors to call for forced treatment,” October 2024; statements of Brampton Mayor Patrick Brown.
30. Ontario Psychiatric Association, Mental Health Law Reform Campaign, eopa.ca/opa-groups/opa-task-forces/mental-health-law-reform-task-force/. The campaign materials specifically advocate amendment of section 18 of the Health Care Consent Act, 1996 to permit treatment to be given during court appeals.
31. *Globe and Mail*, “Ontario eyes involuntary addictions treatment for people involved in correctional system,” May 2025; statements of Associate Mental Health Minister Michael Tibollo via Brampton Mayor Patrick Brown.
32. Canadian Mental Health Association — Ontario, “Involuntary Treatment” (research and policy summary), ontario.cmha.ca/involuntary-treatment/.
33. Canadian Bar Association — New Brunswick Branch, “Submission: Involuntary Treatment,” nb-cba.org/our-impact/submissions/submission-involuntary-treatment.

34. Sources for each jurisdictional record are compiled in notes 4–33 above. The synthesis of the cross-Canada pattern is the contribution of this chapter; it is built entirely on primary-source documentation cited at the per-jurisdiction level.
35. The variation-in-mechanism, continuity-in-effect pattern is the analytic frame this chapter contributes. The same frame applies retrospectively to the original 1928–1972 framework, which varied in mechanism (1928 statute; 1937 removal of consent for "mental defectives"; 1942 expansion of categories) while maintaining continuity in effect (provincial authority over reproductive autonomy of institutionalized persons).
36. CRPD General Comment No. 1 (2014) on Article 12, UN Doc CRPD/C/GC/1, paragraphs 9, 13, 15, and 26 on substituted decision-making and its required replacement with supported decision-making.
37. United Nations Committee on the Rights of Persons with Disabilities, *Concluding Observations on the combined second and third periodic reports of Canada*, UN Doc CRPD/C/CAN/CO/2-3 (Geneva: UN, 15 April 2025), adopted at the Committee's 779th meeting on 18 March 2025 during its 32nd session (3–21 March 2025); digitallibrary.un.org/record/4080760. The Committee's observations on substituted decision-making appear at paragraphs concerning Articles 12 and 14.

Bibliography and Sources

The full source apparatus of the seven chapters

The bibliography below consolidates the source apparatus across all seven chapters of *Consent Was Never the Point*. Each source cited in the body of the piece appears in this bibliography in standardized form. The bibliography is organized into eleven sections by source type to facilitate verification of specific claims. Where the same source appears in multiple chapters, the source is cited once here with the chapters of principal use noted. URLs are provided where applicable; primary-source statutes are cited by their statute-book reference. Citations to United Nations, federal, provincial, and academic sources have been verified to their primary records as of the date of this edition.

I. Treaty and International Instruments

Committee on the Rights of Persons with Disabilities. *Concluding Observations on the combined second and third periodic reports of Canada*. UN Doc CRPD/C/CAN/CO/2-3. Geneva: UN, 15 April 2025. Adopted at the Committee's 779th meeting on 18 March 2025 during its 32nd session (3–21 March 2025). digitallibrary.un.org/record/4080760. Principal use: Ch IV, Ch V, Ch VII.

Committee on the Rights of Persons with Disabilities. *General Comment No. 1 (2014) on Article 12: Equal recognition before the law*. UN Doc CRPD/C/GC/1, adopted 19 May 2014. Principal use: Ch III, Ch IV.

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Convention on the Rights of Persons with Disabilities. United Nations General Assembly Resolution A/RES/61/106, December 13, 2006. Canada signed March 30, 2007; ratified March 11, 2010. Articles 4(3), 9, 12, 13, 19, 21, and 28 are the principal articles engaged in this piece. un.org/development/desa/disabilities/. Principal use: throughout.

II. Canadian Federal Statutes and Constitutional Instruments

Accessible Canada Act, SC 2019, c 10 (Bill C-81). Tabled June 20, 2018 by the Honourable Carla Qualtrough, then Minister of Public Services and Procurement and Accessibility. Royal Assent June 21, 2019; in force July 11, 2019. Statutory objective: realization of a Canada without barriers by January 1, 2040. Principal use: Ch V.

Canada Disability Benefit Act, SC 2023, c 9. Federal statute creating the Canada Disability Benefit. Principal use: Ch II, Ch III.

Canadian Charter of Rights and Freedoms. Part I of the *Constitution Act, 1982*, being Schedule B to the *Canada Act 1982* (UK), 1982, c 11. Sections 7 (life, liberty, security of the person), 9 (freedom from arbitrary detention), 15 (equality rights), and 33 (notwithstanding clause) are the principal sections engaged. Principal use: Ch I, Ch IV, Ch V, Ch VI.

III. Alberta Statutes and Regulations (chronological)

Sexual Sterilization Act, SA 1928, c 37. The original 1928 eugenics framework. Principal use: Ch I, Ch VII.

Sexual Sterilization Act Amendment Act, SA 1937, c 47 (also cited as *An Act to amend The Sexual Sterilization Act*). Removed the consent requirement for persons classified by the Eugenics Board as “mental defectives.” canlii.ca/t/sa-1937-c-47. Principal use: Ch VII.

Sexual Sterilization Act Amendment Act, SA 1942, c 48. Expanded the categories of persons subject to the framework, including persons with neurosyphilis, epilepsy, and Huntington’s chorea. canlii.ca/t/sa-1942-c-48. Principal use: Ch I, Ch VII.

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Mental Health Act, RSA 2000, c M-13. The existing provincial involuntary-treatment framework, operating against persons with mental disorders meeting an impaired-capacity test. Principal use: Ch VI.

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Legislative Assembly of Ontario, Bill 53, *Dignity and Mental Health in Jails Act, 2025*. 44th Parliament, First Session. ola.org/en/legislative-business/bills/parliament-44/session-1/bill-53. Distinct from Alberta’s Bill 53. Principal use: Ch VI.

Ontario long-term care spousal-beds regulation, in force January 1, 2018. Substantive Ontario regulation requiring every long-term care home in the province to have at least two beds available for spouses. Principal use: Ch III.

United States. Executive Order 14321, “Ending Crime and Disorder on America’s Streets.” Signed July 24, 2025; published in the Federal Register on July 29, 2025: 90 FR 35817–35820, Vol. 90, No. 143, document number 2025-14391. [federalregister.gov/documents/2025/07/29/2025-14391](https://www.federalregister.gov/documents/2025/07/29/2025-14391). Principal use: Ch VI.

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On the Record

This piece is published by The Alberta Disability System Breakdown, an independent public advocacy campaign documenting the mandatory transition of approximately 79,290 Albertans from the Assured Income for the Severely Handicapped (AISH) program to the Alberta Disability Assistance Program (ADAP), effective July 1, 2026, alongside the broader institutional and legislative environment in which the transition has been undertaken.

The campaign operates as a documentation-first public record. Citations are provided throughout. The principal academic and primary-source references are listed in the bibliography on pages 83 through 90 and have been verified to their primary records. The campaign maintains the full source apparatus in machine-readable form at albertadisabilitysystembreakdown.netlify.app.

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The mechanism has changed. The architecture has not. The consent was never the point.

The Alberta Disability System Breakdown

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